Twinkle Children's Hospital

User guide

For the front desk, the doctor, and the pharmacy counter.

Every screenshot is taken from the running application. To refresh them all after a change, run the one test that produces them:

```
dotnet test tests/Pharma.UiTests --filter ScreenshotCapture
```

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Common tasks, step by step — the short version of everything above. Start here if you just want to get through a day.

Common tasks, step by step

Each task below is complete on its own. Screen-by-screen detail follows in sections 1 to 16.

A. Set the clinic up — once, before anything else

1. Click **Settings**.

2. **Clinic tab** — clinic name, address, phone. Tick **Registered for GST** only if the clinic actually is, then enter the **GSTIN**; leave it unticked and OPD documents print as a plain document. Click **Save Clinic details**.
3. **Pharmacy tab** — pharmacy name, address, phone, the same GST tick, the **drug licence number** and the **pharmacist's name**. Click **Save Pharmacy details**.
4. **Doctors tab** — enter the first **doctor**: name, speciality, registration number, usual fee. Click **Save doctor**.

You cannot book a visit until at least one doctor exists.

Switch on **Diagnostics** under the **Features** tab too, if this clinic runs its own lab tests — see task J.

B. Book a walk-in patient

1. Click **OPD**, then **+ New visit**. The booking form opens over the screen.
2. Type the patient's **name or phone number** and press Enter.
3. If they are listed, **click the right person**. If a whole family shares the phone, everyone on it appears — pick the child who is actually here.
4. If nobody matches, the new-patient form opens. Enter **name, phone, age, sex**.
5. Choose the **doctor**, adjust **time** and **fee** if needed.
6. Type the **complaint** if you want it on the tile and the prescription.
7. Click **Book visit**. A token number is allocated.

C. Take the consultation fee

1. On the **OPD** screen, find the patient's tile in **Waiting**.
2. Click **Fee** on the tile. A small form opens over the screen.
3. Check the **token, name, age and doctor** at the top — this is the moment to notice you have the wrong child.
4. The **fee** is already filled in from the booking. Change it if you are taking something else — a follow-up at half fee, a family concession.
5. Set **Paid by** — Cash, UPI or Card.
6. For UPI or Card, optionally note the **Transaction / reference no.** — never required, just handy for reconciling against the gateway or bank statement.
7. Leave **Print the receipt** ticked unless you do not want paper.
8. Click **Take fee**. It asks once more, naming the amount and the child. Answer **Yes**.

The badge changes to **Fee paid**. Clicking Fee twice does nothing.

Nothing is written until you answer Yes. Cancel at any point and no receipt number is used up. This matters because a receipt is numbered and dated the moment it is written — a fee taken wrongly has to be reversed on paper, so the software makes you look at it first.

D. See a patient and write a prescription

The consultation is three tabs: **Vitals**, **Prescription**, **Diagnosis**.

1. Click **Consult** on the patient's tile.
2. **Vitals tab** — weight, BP, temperature, height, heart rate, SpO2. All optional.
3. **Prescription tab** — for each medicine:
    - Type two letters or more into **Medicine**.
    - **Click a match** to use one you stock, or keep typing for one you do not.
    - Enter the **dose**.
    - Set **Morning · Afternoon · Night** from the three lists — `0`, `1/4`, `1/2`, `1` or `2`. One in the morning and one at night is `1`, `0`, `1`.
    - Enter **days**.
    - **Qty is filled in for you** in individual tablets — change it if you want.
    - Click **Add**. The whole row then empties, dose and days included, ready for the next medicine.
4. **Diagnosis tab** — complaint, diagnosis, advice, a review date if there is one, and, if the **Diagnostics** module is switched on, any lab tests the patient needs. Search or type a test and click **Add** to put it on the list — this does not bill anything; it is picked up later at the Diagnostics desk. See section 11.
5. Click **Save & complete**. The tile moves to **Completed**.

The consultation covers the whole window while it is open, and the rest of the app waits behind it. Nothing else can be started until you leave, so a consultation cannot be opened and then forgotten. **Close** or the **Esc** key leaves it; if anything has been typed and not saved you are asked first.

E. Add a new medicine and put stock on the shelf

This is two jobs on two screens. **Medicines** is where a medicine is described, once. **Inventory** is where stock arrives, every delivery.

First, describe the medicine — Medicines screen

1. Click **Medicines**, then **+ New medicine**. The form opens over the screen. To change one already there, select the row and click **Edit** — or just double-click it.
2. Enter the **brand name** as printed on the pack — *Calpol*.
3. Enter the **drug / generic name** — *Paracetamol 250mg*. Staff search by either, so filling both is worth the few seconds.
4. Enter the **manufacturer** and **pack size** as printed.
5. Set **Sold as** — tablet, capsule, bottle, sachet.
6. Set **Units in one pack** — `15` for a strip of fifteen, `1` for a bottle.
7. Set **GST %**, **schedule**, **rack** and **reorder level**.
8. Click **Save medicine**. The form closes — that is normal. The message underneath the catalogue confirms what was saved.

Then, put stock on the shelf — Inventory screen

9. Click **Inventory**, type the name in the search box and click **Search**.

10. Click the medicine in the list. The heading names it and says how much is on hand.
11. Click **Receive stock**. A form opens over the screen, headed with the medicine's name and what is already on the shelf.
12. Enter **batch no**, **expiry**, **packs**, any **free** packs, the **rate** you paid and the **MRP** printed on the pack.
13. Check the line that appears — "*20 pack(s) × 15 = 300 tablets onto the shelf*".
14. Click **Add stock**. The form closes and the screen clears, ready for the next line of the delivery note.

If you are entering several medicines off one delivery note, repeat steps 9–14 for each.

Everything clears between lines, the supplier and bill number included. That is deliberate. A supplier left in the box is a supplier silently attached to the next delivery, and a wrong supplier on a batch is worse than a blank one — it is wrong in the reconciliation report rather than merely missing from it.

F. Load a supplier's file instead of typing it

1. Click **Inventory**, then **Import supplier bill**.
2. Choose the **supplier profile** that matches their format.
3. Click **Browse…** and pick the file. It is read immediately.
4. Type the **supplier name** — the file does not contain it.
5. Check the grid, especially **PER PACK**. `30s` reads as 30; `60ML` stays 1. **Type the real number for any strip.**
6. Read the notices underneath — bill date, MRP changes, anything unclear.
7. Click **Import**.

Stock is **added** to what is already there. The same bill cannot be imported twice.

G. Sell to someone who walks in

1. Click **Pharmacy counter**.
2. Type part of the medicine name. It filters as you type, in any case.
3. **Click the medicine** in the results.
4. Enter **How many**, and check the unit beside it says what you mean — *tablets* or *strips of 10*.
5. Click **Add to bill**. The search box empties and the medicine is let go of — the line is on the bill, so start the next one by typing its name.
6. Repeat 2–5 for each medicine. The bill builds up; only the search clears.
7. Leave the customer as **Guest**, or type their name.
8. Choose **Payment**. For UPI or Card, optionally note the **Transaction / reference no.**
9. Click **Save & print bill**.

H. Sell part of a strip

Exactly as task G. Nine tablets out of a strip of ten is `9` with the unit on **tablets** — the bill line then reads `0 × 10 TAB + 9 tablets` so you can see what to hand over.

The price is proportional: a strip of ten at ₹120 makes each tablet ₹12, so nine is ₹108. **A full strip always costs exactly the MRP printed on it** — the arithmetic never drifts by a few paise.

Switch the unit to **strips of 10** and type `2` when someone wants whole strips; it becomes 20 tablets.

> **If the unit only offers "units"**
>
> That medicine has **Units in one pack** set to 1, so the software believes one strip *is* one tablet — and nine tablets will be charged as nine strips. **Settings** → **Check data health** finds every medicine in that state and fixes them together. See task P.

I. Dispense what a doctor prescribed

1. Click **Pharmacy counter**.
2. Under **Or pull today's OPD prescription**, choose the patient.
3. Click **Load prescription**. Everything prescribed **and in stock** is added with the right quantity.
4. Anything missing is named on screen — tell the parent to buy it outside.
5. Choose **Payment**, then **Save & print bill**.

J. Bill a patient for a lab test

1. Turn the module on once, if it is not already: **Settings** → **Features** → **Diagnostics**, then **Save features**. A **Diagnostics** item appears in the sidebar immediately.
2. Click **Diagnostics**.
3. Search for the patient by **name or phone**, same as everywhere else — or, if the doctor already requested tests for them during today's consultation, skip straight to step 5.
4. Click **+ Add test**, tick every test wanted, **Done**.
5. **Or**, under **Or pull today's OPD test requests**, choose the visit and click **Load diagnostic tests** — this picks the patient and the tests together, in one click.
6. Adjust **Discount** if needed. **Referred by** only appears for a patient who did not come through this clinic's own OPD.
7. Choose **Payment**, then **Save & print bill**.

K. Load the tests a doctor requested during consultation

The lab equivalent of task I.

1. **Diagnostics**, under **Or pull today's OPD test requests**.
2. Choose the visit — it only lists visits with tests requested and not yet billed.
3. Click **Load diagnostic tests**. Every test from that consultation is added, and the patient is selected automatically.
4. Choose **Payment**, then **Save & print bill**.

L. Correct a stock count

1. Click **Inventory** and select the medicine.
2. Click **Correct count**. A form opens over the screen.
3. Choose the **batch**. If there is only one it is already chosen.
4. Enter the **true count** — what is actually on the shelf, in units. It starts at what the system currently believes, so you are changing a number rather than typing one from nothing.
5. Choose a **reason**: recount, breakage, expired, lost, entry error.
6. Add a **note** if it is unusual.
7. Click **Correct count**.

Every correction is recorded in **Recent corrections** at the foot of the Inventory screen, with was, now and why.

If the medicine has nothing on the shelf, it says so rather than opening an empty form — there is no count to put right.

The form closes afterwards and takes the batch with it. Correcting a second batch means opening it again — which is the point: a correction made against a batch you had forgotten was still selected writes off the wrong stock, and the trail then says you meant to.

M. Print something again, however long ago

1. Click **Patients** and search by name or phone.
2. Select the patient.
3. **Visits & prescriptions** tab → select the visit → **Print prescription** or **Print fee receipt**.
4. **Medicine bills** tab → select the bill → **Print bill**.

For a walk-in with no patient record: **Reports** → **Day book** → **Find any bill**, search the bill number or customer name across all dates, then **Reprint**.

N. Close the day

1. Click **Reports**.
2. Check the totals along the top — pharmacy sales, cash, UPI, fees collected.
3. **Day book** — every bill of the day.
4. **GST summary** — only if registered.
5. **Expiring soon** — return these to the distributor.
6. **Low stock** — what to order.
7. **Stock to reconcile** — anything added at the counter without a supplier bill. Match each to the bill when it arrives.
8. **Schedule H1 register** — keep for three years.

O. Sell something the system says you do not have

1. At the **Pharmacy counter**, find the medicine and click it.
2. Click **Stock came in — add it**.
3. Enter **packs on the shelf** and the **MRP**. Everything else is optional.
4. Click **Add to shelf** and carry on with the bill.

It appears under **Reports** → **Stock to reconcile** until you match it to the supplier bill.

P. Put the medicine records right

Do this **once**, before you rely on the counter. It is the difference between nine tablets costing ₹108 and costing ₹1,080.

1. Click **Settings**, then **Check data health**.
2. Read the list. Each row says what is wrong, what it will become, and what happens to the stock figure.
3. Leave the ticks as they are and click **Put the ticked ones right**.

What it finds	What it means
Pack size disagrees	The pack says 15 but the medicine says 1 per pack, so the counter sells whole strips to anyone asking for tablets. Fixing it re-counts 59 strips as 885 tablets — the same packs on the shelf
Old stock at a different pack size	Stock received before the medicine was corrected. Re-counted to match
Sold-as not set	Quantities read "units" instead of tablets. Taken from what is printed on the pack
Duplicate — fix by hand	The same medicine twice. Press **Merge…** on the row

Every re-count writes a line in **Inventory** → **Recent corrections**, so nothing changes quietly.

> ### Before you tick a re-count, check one thing
>
> *"59 → 885"* is right **only if the 59 means 59 strips**. If it means 59 loose tablets, untick that row — it would multiply your stock by fifteen.

Q. Merge a medicine that appears twice

1. **Settings** → **Check data health**.
2. Find the row marked **Duplicate** and click **Merge…**.
3. Confirm.

Its batches, purchases, sales and prescriptions all move to the record holding the most stock, and the empty one is retired. Nothing is deleted.

You will not be able to create a new duplicate — the same brand, maker and pack is refused, and you are offered the existing one instead.

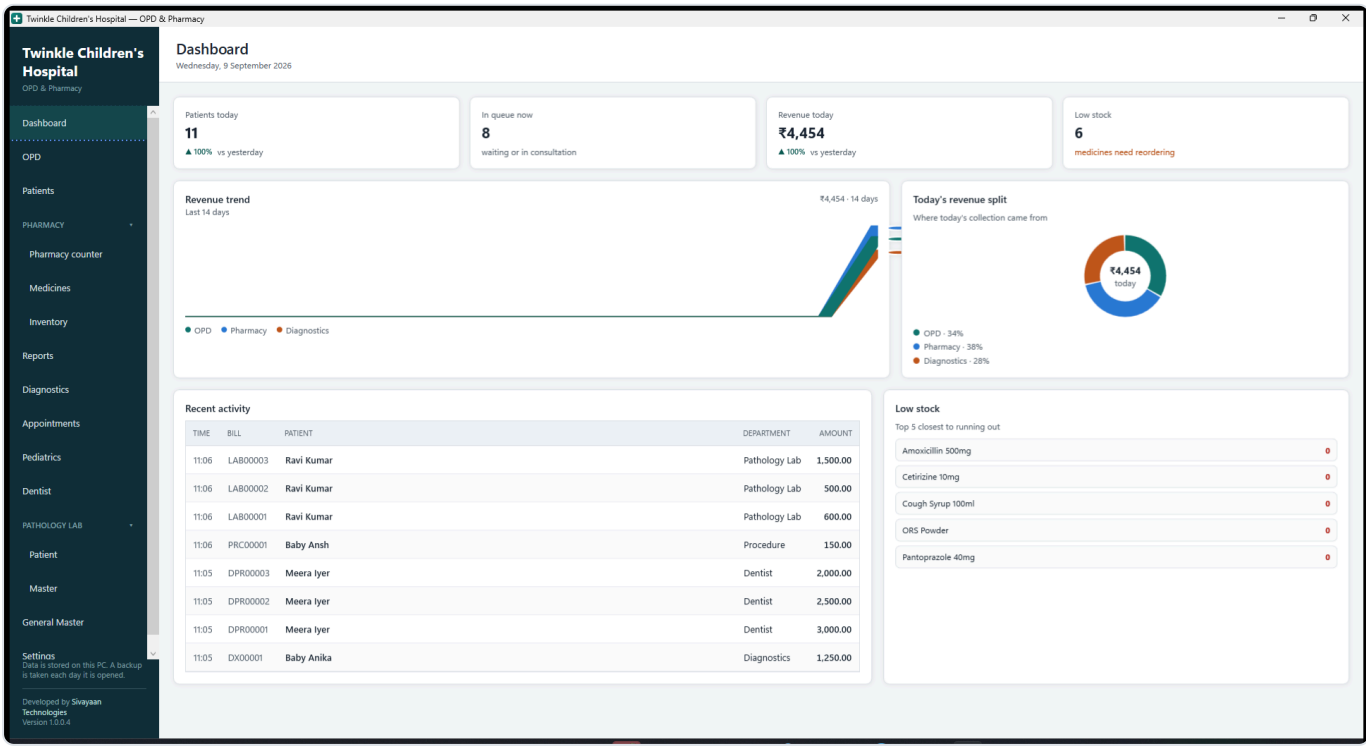
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1. The window

Screens down the left. The one you are on is highlighted. **Dashboard** is what opens first.

Button	What it is for
Dashboard	Today at a glance — see below
OPD	The day's queue. Book visits, take fees, open consultations
Patients	Everyone ever registered, with their whole history
Pharmacy counter	Selling medicines
Medicines	The catalogue — what each medicine *is*. Set up once
Inventory	Stock — what is on the shelf. Receiving, batches, corrections
Reports	End of day, GST, expiry, low stock
Diagnostics	Lab test billing. Only shown once switched on — section 11
Settings	Clinic details, doctors, screen layout, optional modules

The Dashboard



The landing screen — a five-second read of how the day is going, not another place to do work. Every figure on it already has a real screen behind it for the detail.

Card	Shows
The four tiles	Patients and revenue for the chosen period, how many are in queue right now, and medicines running low — the first two against the period before where that means anything
Revenue trend	One line per department — OPD, Pharmacy, Diagnostics — over the period, all three drawn to the same scale so they are honestly comparable. A short period is drawn over at least 14 days so there is a line to read; the figures above stay strictly the period you chose
Revenue split	The same three departments, as a proportion of the period's collection
Recent activity	The period's OPD fees, pharmacy bills and diagnostic bills, newest first
Low stock	The five medicines closest to running out

The Diagnostics line, slice and legend entry only appear once that module is switched on.

Choosing the period

From and **To** at the top right set the period. It opens on **today to today**, exactly as it always has, and **Today** brings it back.

- The heading, the tile labels and the arrows all say which period you are looking at, so a week's total is never mistaken for a day's.
- The arrows compare against **yesterday** for a single day, and otherwise against the stretch of the **same length straight before** — a week against the week before, never a week against yesterday.
- **In queue now** and **Low stock** ignore the period on purpose: who is waiting, and what is running out, are facts about this minute.
- From after To is accepted and read the right way round.

Medicines and Inventory are deliberately separate. Describing a medicine is a one-off job usually done by whoever sets the shop up; receiving stock happens every delivery and is done at the counter. Keeping them apart means neither screen is crowded with the other's fields.

The application opens **maximised**, filling the screen. The screens it runs on are small, and every pixel spent on desktop around the edge is a pixel not spent on the queue.

Forms open over the screen

Anything you fill in opens **over** the screen you were on, greyed out behind it, and the rest of the app waits until you are done:

- Booking a visit, and taking the fee
- Adding or editing a patient
- Adding or editing a medicine
- Receiving stock, and correcting a count
- The consultation, importing a supplier bill, a print preview

This is on purpose, and it is worth knowing why. These used to be columns down the right-hand side of each screen, permanently there and never empty. On a small screen that meant scrolling to reach the Save button — and a form you scroll to save is one people abandon half-filled. Worse, a column that is never empty always has *something* in it from the last time, which is how a patient gets saved on top of another one.

A form over the screen is opened for one job and taken away when that job is done. **Cancel**, or the **Esc** key, closes it and writes nothing.

That is also why the lists behind them now have the whole window: the catalogue, the register and the stock list are no longer sharing it with a form.

The heading of every screen shows the screen name and a one-line summary underneath — for example *"2 waiting · 1 completed · Sun, 26 Jul"*.

At the bottom left it reminds you where the data lives and that a backup is taken each day the application is opened. Under that is who built the software and which version you are running:

> Developed by Sivayaan Technologies Version 1.0.0.4

Read that version number out if you ever ring for help — it says which build of the software you have, which is the first thing anyone helping you needs to know. It is on screen whatever page you are on, so nobody has to go looking for it.

Screens clear themselves when the job is done

Whenever you finish something — save a patient, save a medicine, add a line to a bill, put stock on the shelf — the screen empties itself, **including the search box**, and the message underneath tells you what was saved.

This is deliberate and it matters. A form that still holds the last record is not ready for the next one: typing the next patient's name into it and pressing Save would change the person you just entered instead of adding anybody. The same at the counter — the medicine you just billed would still be selected, and pressing Add again would re-do the line you already have.

So after every save you start from a clean screen. If you want to look at what you just saved, search for it again; it is there.

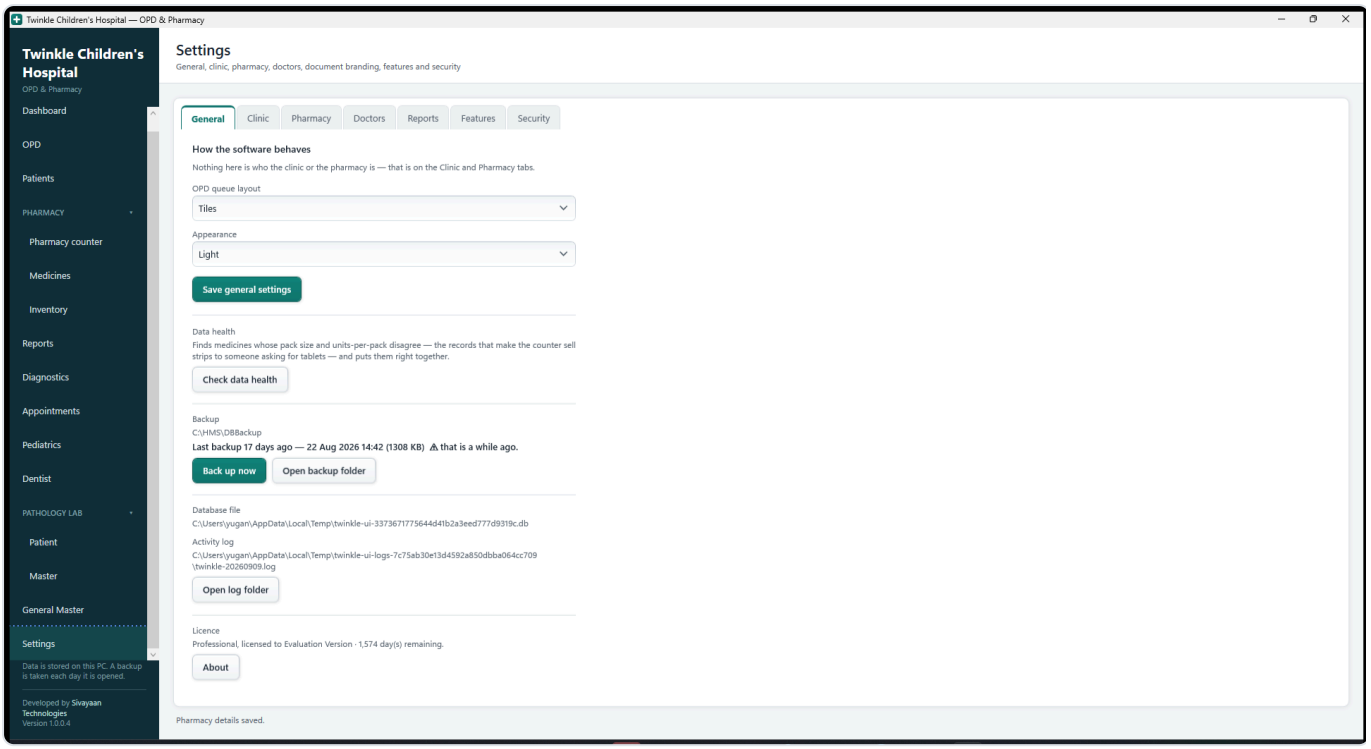
For the forms that open over the screen, "clear" means the form has **gone**, taking everything typed into it. There is nothing left behind to be saved by accident, because the next one gets a fresh form.

Nothing carries over between deliveries either — not the supplier, not the bill number. Those two used to stay, on the reasoning that one delivery note covers many medicines. In use that was the wrong trade: a supplier left in the box is a supplier silently attached to the next delivery, and a wrong supplier on a batch is worse than a blank one.

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2. Settings — set this up first

Everything here prints on your bills, receipts and prescriptions. Six tabs: **General**, **Clinic**, **Pharmacy**, **Doctors**, **Reports**, **Features**.

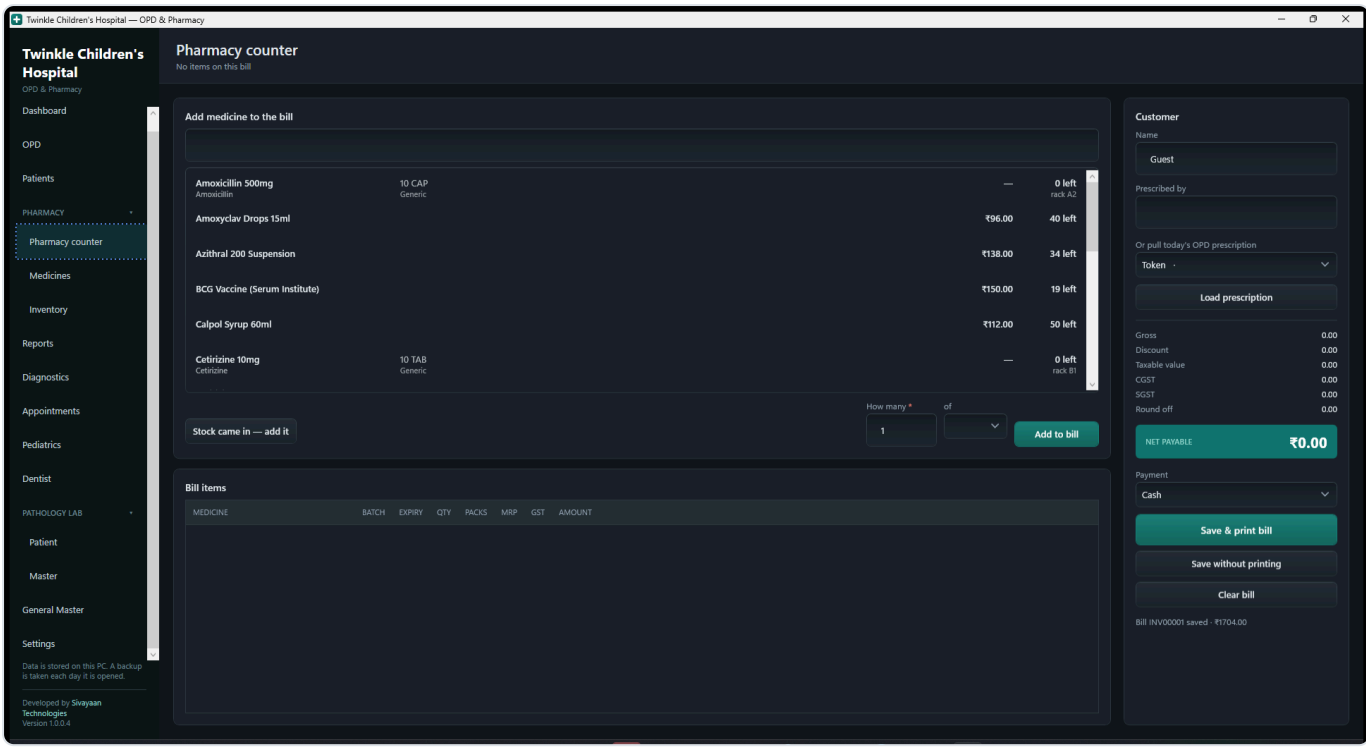


General

Not who the clinic or pharmacy is — that is the Clinic and Pharmacy tabs. This tab is how the software behaves, plus housekeeping.

Control	What it does
OPD queue layout	`Tiles` or `Rows` — see section 3
Appearance	`Light` or `Dark`. Changes immediately; **Save general settings** makes it stick for next time
Check data health	Finds medicines whose pack size and units-per-pack disagree, and puts them right — see task P
Back up now	An extra backup beyond the automatic daily one. Shows where backups are kept and when the last one ran
Database file, **Activity log**	Exact paths, with an **Open log folder** button. You need these only when reporting a problem
Licence	What you are licensed for, and how long is left on it

Printed bills, receipts and prescriptions are always black on white whatever you choose for Appearance. Paper is paper.



Clinic

Everything printed on an OPD prescription and fee receipt.

Control	What to enter
Clinic name	Printed largest, at the top of every OPD document
Address line 1 / 2, **Phone**	Under the name
Registered for GST	Off by default — see below
GSTIN	Only enabled when registered
Consulting hours	When the doctor sits, morning and evening. See below
Footer / disclaimer	Free text at the bottom of a prescription, e.g. "Get well soon"
Save Clinic details	New documents use it immediately

Consulting hours

Most clinics run two sittings with the afternoon off. Set the hours here and the OPD screen can show one sitting at a time:

```
Morning   10:00  to  13:00
Evening   16:00  to  20:00
```

Use the 24-hour clock. The line underneath reads the four boxes back as a sentence, so a typo is caught before it is saved rather than the next time somebody wonders where the queue went. The end of a sitting is **exclusive** — a morning ending at 13:00 does not also claim the one o'clock patient.

These are only a filter on the OPD screen. Nothing stops you booking a visit outside them, and a visit booked outside both sittings still shows on **Full day** — see section 3.

> The application works with these blank — the GSTIN line simply does not print. Fill them in before you issue a real bill to a customer.

> ### Registered for GST, or not
>
> This one setting changes what your bills legally are.
>
> | | Registered **off** | Registered **on** |
> |---|---|---|
> | Bill is headed | `INVOICE` | `TAX INVOICE` |
> | GSTIN printed | no | yes |
> | GST charged | **none** | extracted from the MRP |
> | GST column and summary | hidden | shown |
>
> It is **off by default on purpose**. Issuing a document headed "tax invoice", with a GSTIN on it, when you are not registered is a false statement — so switching it on has to be a deliberate act.
>
> Turning it on later does not rewrite old bills. Each bill remembers what it was when it was issued, so a reprint always shows what the customer was given.

Pharmacy

Everything printed on a medicine bill.

Control	What to enter
Pharmacy name	Printed largest, at the top of every pharmacy bill
Address line 1 / 2, **Phone**	Under the name
Registered for GST	Off by default — same rule as the Clinic tab, see above
GSTIN	Only enabled when registered
Drug licence no	Your 20B/21B number. **Required on a chemist's bill**
Pharmacist	Printed at the foot of the bill
Footer / disclaimer	Free text at the bottom of a bill

Control	What to enter
Save Pharmacy details	New bills use it immediately

Doctors

Control	What it does
List	Every doctor. Click one to edit it
Name	Appears as an OPD tab and on the prescription
Speciality	Printed under the doctor's name on the prescription
Registration no	Printed on the prescription. Required on a real one
Phone (optional)	Not printed
Default consultation fee	Fills in automatically when booking for this doctor
Qualification (optional)	Degrees, printed beside the name on the receipt — e.g. `MBBS, MD (Paediatrics)`
OPD valid for (days)	How long one fee covers this doctor. `0` charges every visit
Save doctor	Saves the one being edited, then clears the form
Clear	Empties the form. Saved doctors are unchanged
+ New doctor	Clears the form to add another

At least one doctor is needed before any visit can be booked.

OPD valid for — one fee, several visits

Most clinics let a patient come back for a short while on the fee they have already paid. Set **OPD valid for** to 7 and a patient who pays this doctor today may see them again free for the next seven days.

- It is **per doctor**, because it is that doctor's own money being given away. A fee paid to one doctor never buys a free visit with another.
- It is counted in **whole days**, never hours. Paid at 9pm on the 1st with seven days set, they are still covered all day on the 8th.
- The window is fixed **when the money is taken**, and printed on the receipt. Changing this number later does not shorten or extend a window already given to somebody — they were told a date, and that date stands.
- Only a payment opens a window. A free review does **not** start a new one, or a patient returning every sixth day would never pay again.
- `0` — the default, and what every doctor already on file stays at — turns the whole thing off and charges every visit.

At the desk this is automatic. Book a patient who is still inside the window with the same doctor and the booking form says so, and sets the fee to zero:

> Review — ₹300 paid on 02 Sep (receipt RCP00041), free with this doctor until 09 Sep 2026.

The fee box stays editable. A patient returning inside the window but for a genuinely new complaint is usually still charged, and that is the desk's call to make — type the fee back in and the visit is recorded as New and paid, exactly as if there had been no window at all.

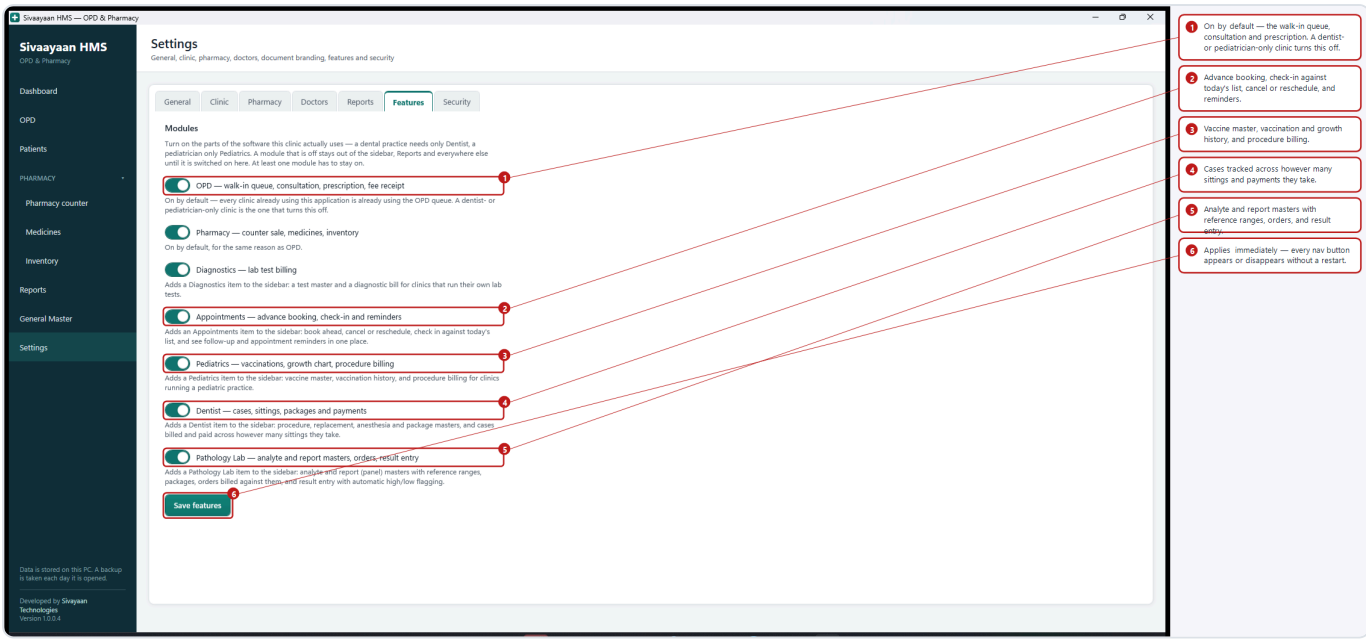
Reports — document branding

Despite the name, this tab is about how documents look, not the Reports screen. It is the shared branding behind every prescription, receipt and bill.

Control	What it does
Logo	Printed at the left of the header, beside the clinic or pharmacy name. PNG or JPEG, under 1 MB, wide rather than square works best. No logo prints the name centred in text only
Upload logo / **Remove logo**	
Bottom message	Printed at the foot of every document — a returns policy, a thank-you
Save document branding	

Features

Which parts of the software this clinic actually uses. A module that is off stays out of the sidebar, Reports and everywhere else until it is switched on here — not just hidden. **At least one module has to stay on.**



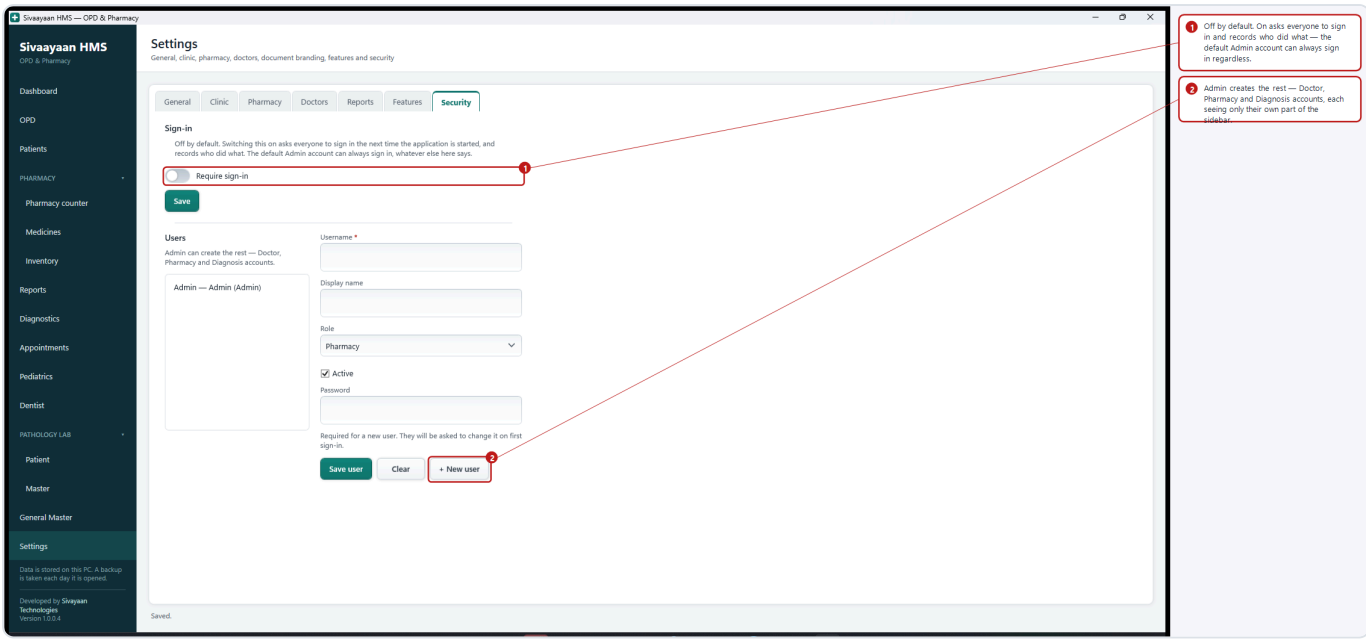
	Control	What it does
1	**OPD**	The walk-in queue, consultation, prescription and fee receipt. On by default — a dentist- or pediatrician-only clinic is the one that turns this off
2	**Appointments**	Adds **Appointments**: book ahead, check in against today's list, cancel or reschedule, and reminders — section 19
3	**Pediatrics**	Adds **Pediatrics**: vaccine master, vaccination and growth history, procedure billing — section 20
4	**Dentist**	Adds **Dentist**: cases billed and paid across however many sittings they take — section 18
5	**Pathology Lab**	Adds **Pathology Lab**: analyte and report masters with reference ranges, orders, result entry — section 21
6	**Save features**	Applies immediately. Every nav button appears or disappears without a restart

Not pictured above: **Pharmacy** (counter sale, medicines, inventory — on by default, same reason as OPD) and **Diagnostics — lab test billing** (adds **Diagnostics**: a test master and diagnostic billing — section 11), both further up the same list.

Pediatrics and Dentist share a **Vaccine Master** and a **Procedure Master** — see General Master — rather than each keeping its own separate copy.

Security

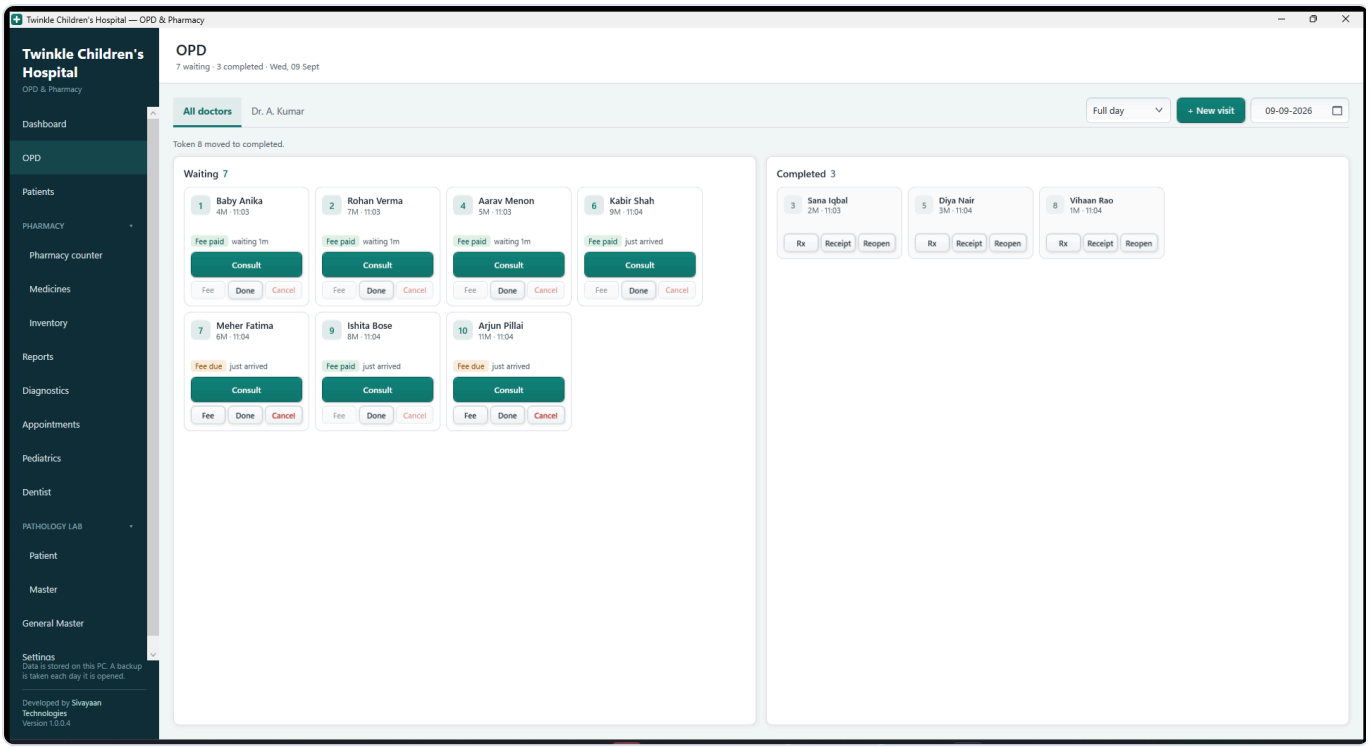
Off by default. Switching **Require sign-in** on asks everyone to sign in the next time the application is started, and records who did what against every bill, prescription and record from then on. The default **Admin** account can always sign in, whatever else here says.



	Control	What it does
1	**Require sign-in**	Off by default — every user is treated as one shared front desk until this is on
2	**+ New user**	Admin creates the rest — Doctor, Pharmacy and Diagnosis accounts, each role seeing only its own part of the sidebar

A new user is asked to change their password on first sign-in. Signing in adds a **logged in as** name and a **Log out** button to the bottom of the sidebar; signing out returns to the sign-in screen without closing the application.

3. The OPD screen



Top row

Control	What it does
All doctors tab	Shows every patient in the clinic
Doctor tabs	One per doctor. Shows only their patients
Sitting	`Full day`, `Morning` or `Evening`. See below
+ New visit	Opens the booking form over the screen
Date	Which day you are looking at. Defaults to today

The doctor is a tab rather than a column on each patient, which is why the tiles stay small.

Morning, evening or the full day

Doctors sit mornings and evenings with the afternoon off, so *"who is left this evening"* is usually the real question. The picker narrows both columns to one sitting; the hours come from **Settings → Consulting hours**.

The heading then names the sitting and its hours:

> 3 waiting · 1 completed · Fri, 31 Jul · **Morning sitting, 10:00 to 13:00**

It tells you when it is hiding somebody. A visit booked at two in the afternoon belongs to neither sitting, so on **Evening** the heading reads:

> 0 waiting · 0 completed · Fri, 31 Jul · Evening sitting, 16:00 to 20:00 · **1 more today outside these hours**

Switch to **Full day** and everybody is there. Full day is how the screen starts, and it hides nobody — so if a patient seems to have vanished, that is the first thing to check.

The two columns

Waiting — everyone still to be seen. **Completed** — everyone finished.

A patient moves from one to the other and can be moved back.

What a waiting tile shows

On the tile	Meaning
Green number	**Token number**. What you call out
Name	The patient
`4F · 08:55`	Age, sex, and the time they were booked
Grey line	What they came in with
`Fee paid` / `Fee due`	Green if the consultation fee is taken, amber if not
`just arrived` / `waiting 12m`	How long they have been sitting there

Buttons on a waiting tile

Button	What it does
Consult	Opens the consultation window for this patient
Fee	Opens the fee form — see section 5. Greyed out on a review, which owes nothing
Done	Moves the tile to Completed without a consultation
Cancel	Cancels the visit. Asks first. Greyed out once the fee has been taken or the visit is finished — see below

Buttons on a completed tile

Button	What it does
Rx	Prints the prescription. Says so if there isn't one
Receipt	Prints the fee receipt again, marked DUPLICATE

Button	What it does
Reopen	Moves the patient back to Waiting

When a visit can no longer be cancelled

Cancel is for a booking made in error, and it stays available only while nothing has happened yet. The button goes grey — and the application refuses the change even if you reach it another way — once either of these is true:

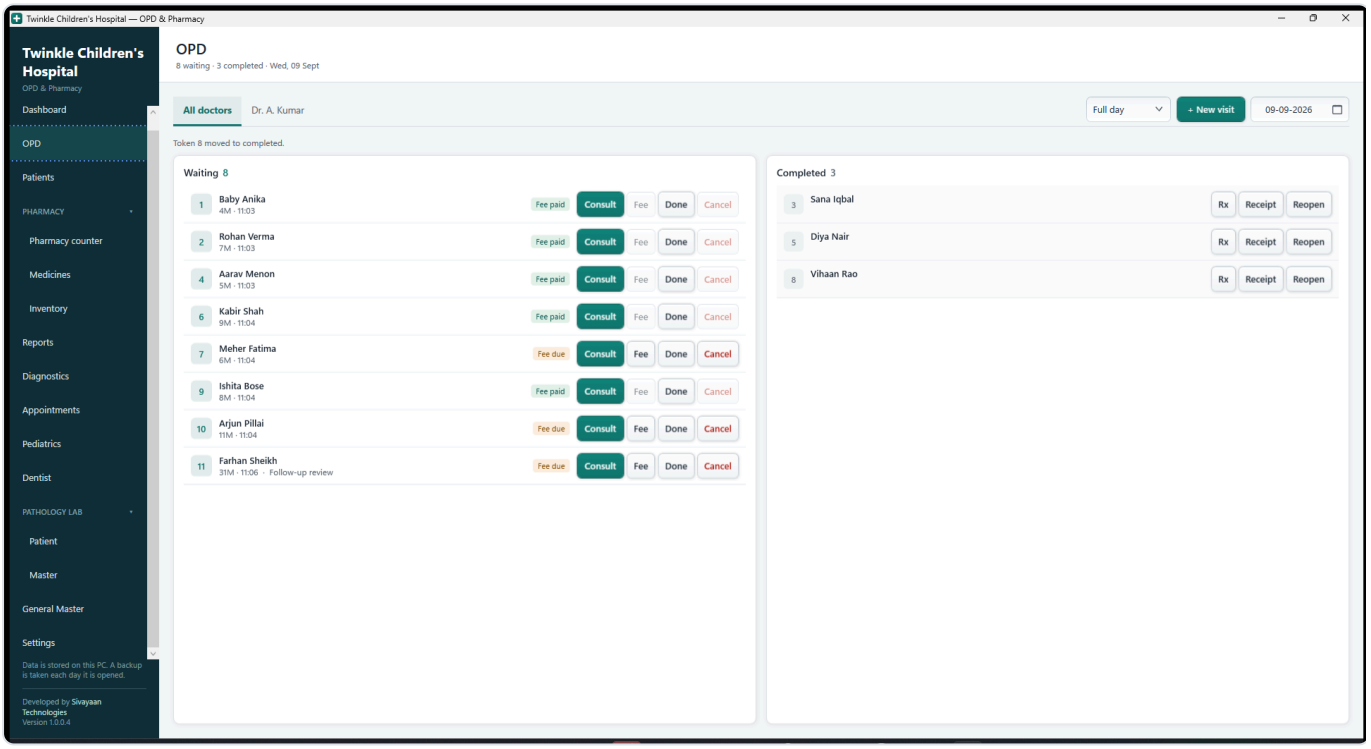
- **The fee has been taken.** There is a numbered receipt in the patient's hand. Cancelling would leave that receipt pointing at a visit the register says never happened.
- **The visit is completed.** The doctor has finished and the consultation is on the record.

A patient who has paid and then leaves is not a cancellation. Mark the visit **Done**, and handle the money the way you handle any other refund at the desk. The register keeps showing what actually happened, which is what the day book and the reports are counting.

A visit that is only part-way through — the patient is in the room, nothing paid — can still be cancelled.

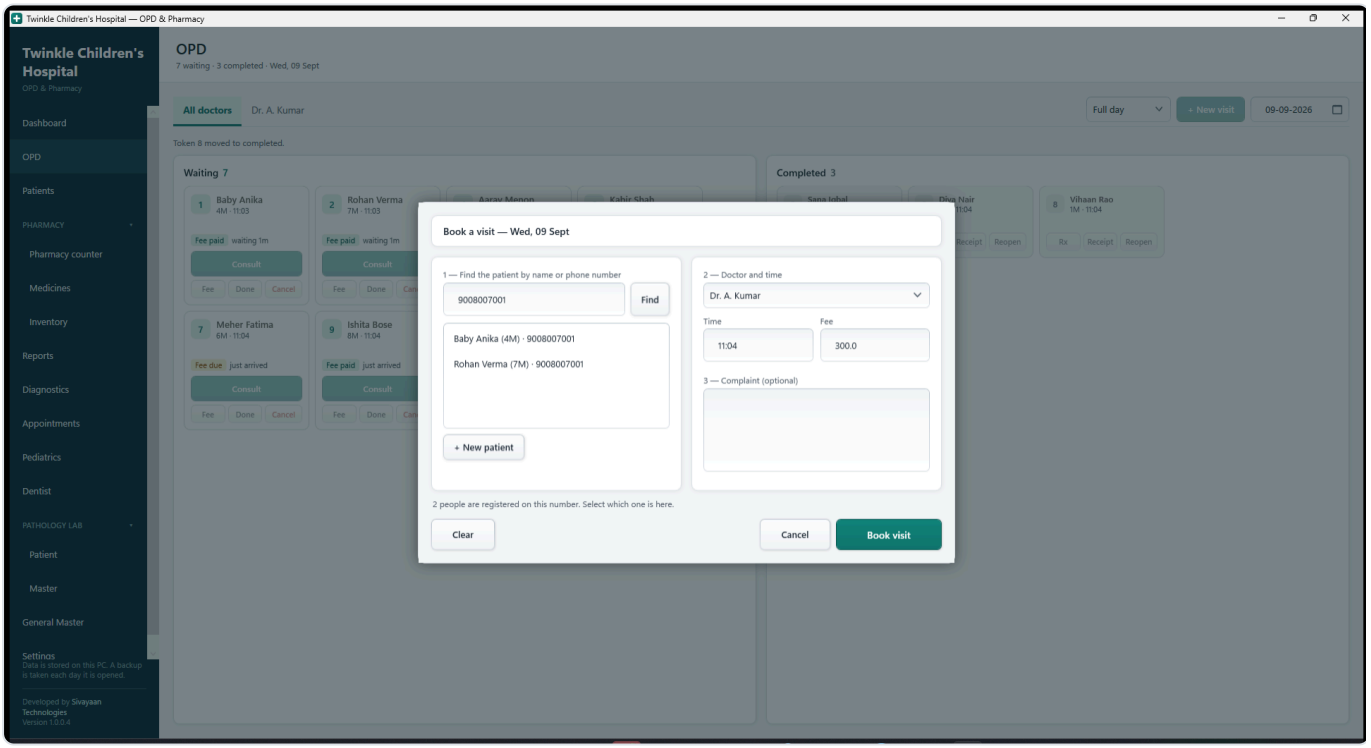
Choosing tiles or rows

Set in Settings. Tiles are easier to read across a room; rows fit more people on screen when the clinic is busy. **Everything works the same in either.**

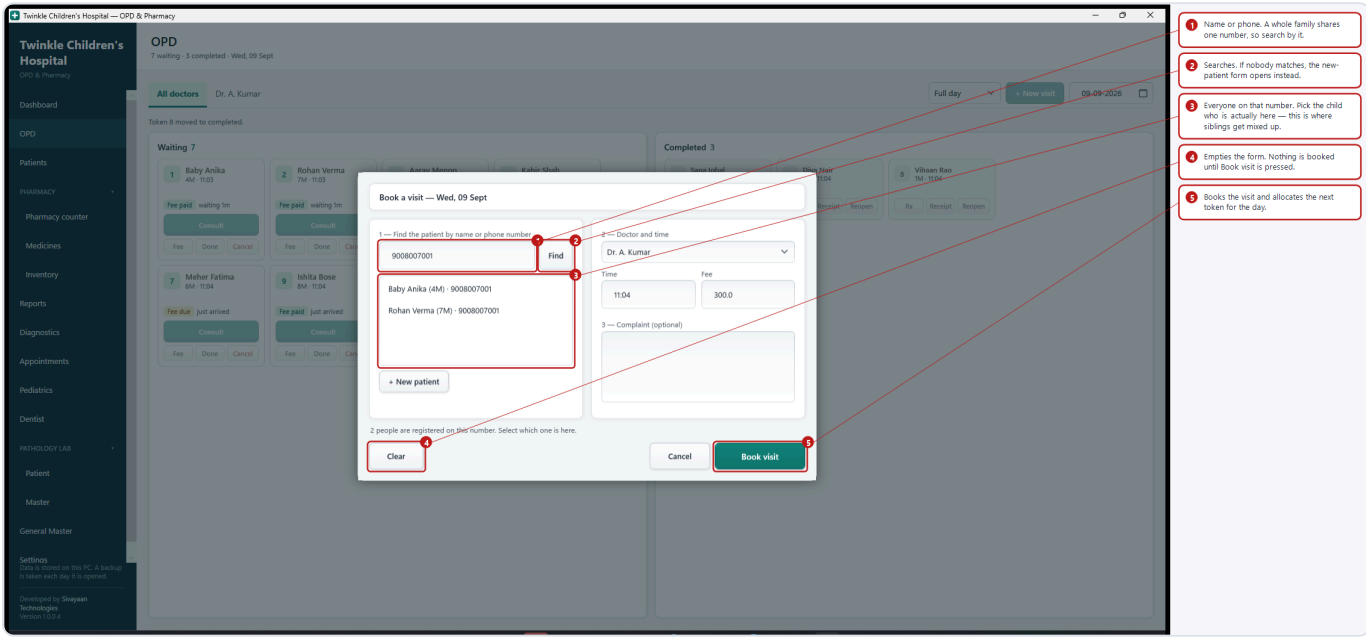


4. Booking a visit

Click **+ New visit**. The form opens over the screen, in three numbered steps — who is here on the left, when and why on the right.



Every control, ringed and explained:



Step 1 — find the patient

Control	What it does
Search box	Type a name or a phone number. Enter also works
Find	Runs the search
Results list	Everyone matching. **Click the right one**
+ New patient	Opens the form below to register someone new

If nobody matches, the new-patient form opens by itself with what you typed already filled in.

New patient field	Notes
Name	The only one that is required
Phone	The parent's number. Shared across siblings is normal
Years	The age in years. Use **0** for a baby under one
Months	Optional. Blank and **0** mean the same — a whole number of years
Sex	Male, Female or Other
Back to search	Returns to the list without adding anyone

Step 2 — doctor and time

Control	Notes
Doctor	Defaults to whichever tab you were on
Time	24-hour, e.g. `09:45`. Defaults to now
Fee	Defaults to that doctor's usual charge

Step 3 — complaint

Optional. Whatever you type shows on the queue tile and on the prescription.

Then **Book visit**. A token number is allocated automatically and the form closes. **Clear** empties the form without closing it; **Cancel** closes it and books nothing.

The payment method is no longer chosen here — it is asked for when you actually take the money, in section 5.

One phone, several children

A parent's number covers the whole family. Typing it lists **every child** registered against it, and the message says so: "*3 people are registered on this number. Select which one is here.*"

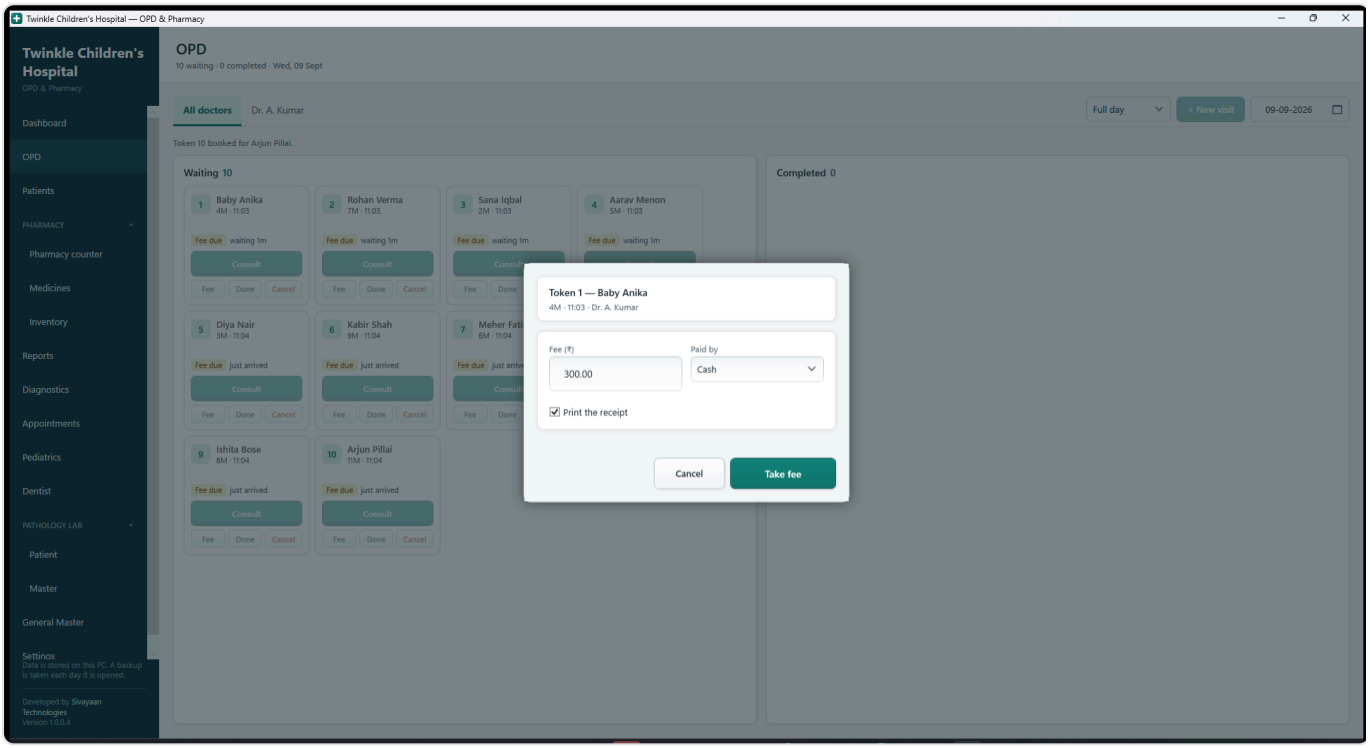
The number is matched on its digits, so `9008007001`, `+91 90080 07001` and `90080 07001` all find the same family.

If you click **Book visit** without choosing one, the application **stops you** rather than quietly registering a fourth child who already exists.

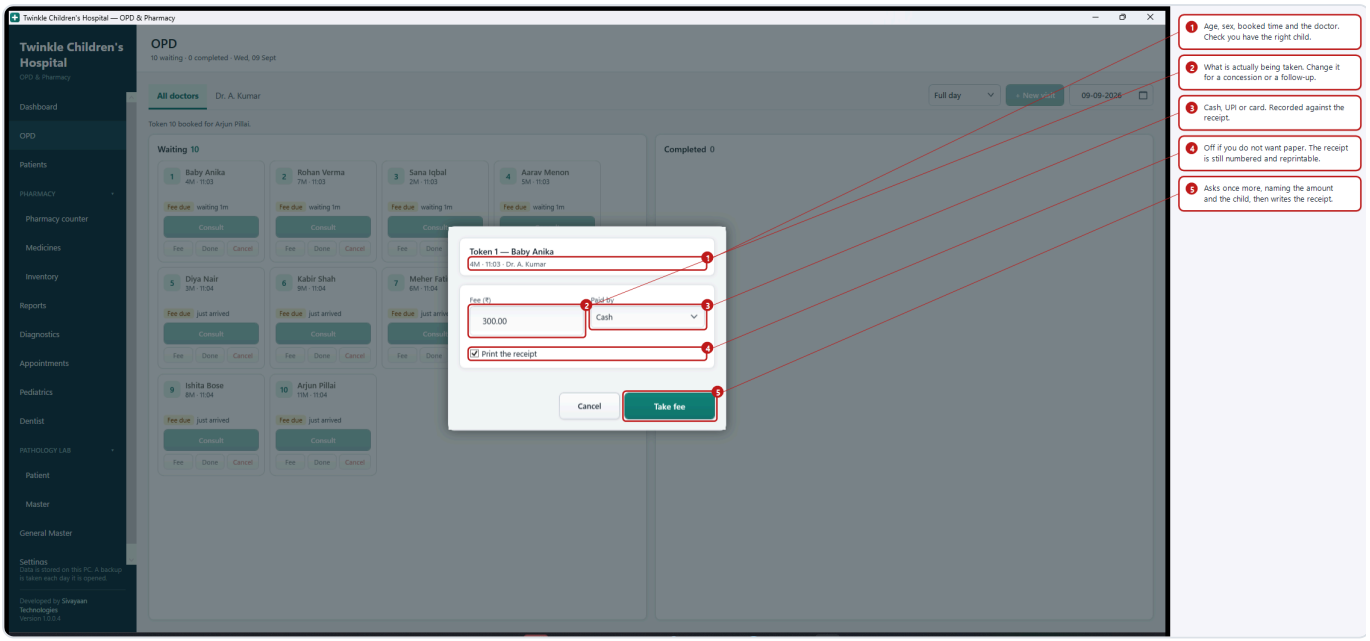
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5. Taking the consultation fee

Click **Fee** on a waiting tile. A small form opens over the screen.



Every control, ringed and explained:



Control	What it does
Heading	Token number and the patient's name
Line underneath	Age, sex, the booked time and the doctor
Fee (₹)	What you are actually taking. Filled in from the booking
Paid by	Cash, UPI or Card. Recorded against the receipt
Print the receipt	On by default. Turn it off if you do not want paper
Cancel	Closes it. Nothing is taken and no receipt number is used
Take fee	Asks once more, then writes the receipt

Changing the amount

The fee starts at whatever was quoted when the visit was booked. Change it for a follow-up seen at half price, a family concession, or a rounding down. A note appears the moment it differs:

> Booked at ₹300.00. This receipt will say ₹150.00.

That note is there because a concession is a decision and a mistyped digit is not, and on screen the two look identical.

The confirmation

Take fee does not take the fee. It asks:

> Take ₹300.00 from Baby Anika by Cash?

Only **Yes** writes anything. This is the last gate before a receipt number is burnt, and it names the three things that get mixed up when two people are at the desk at once — the amount, the method and the child.

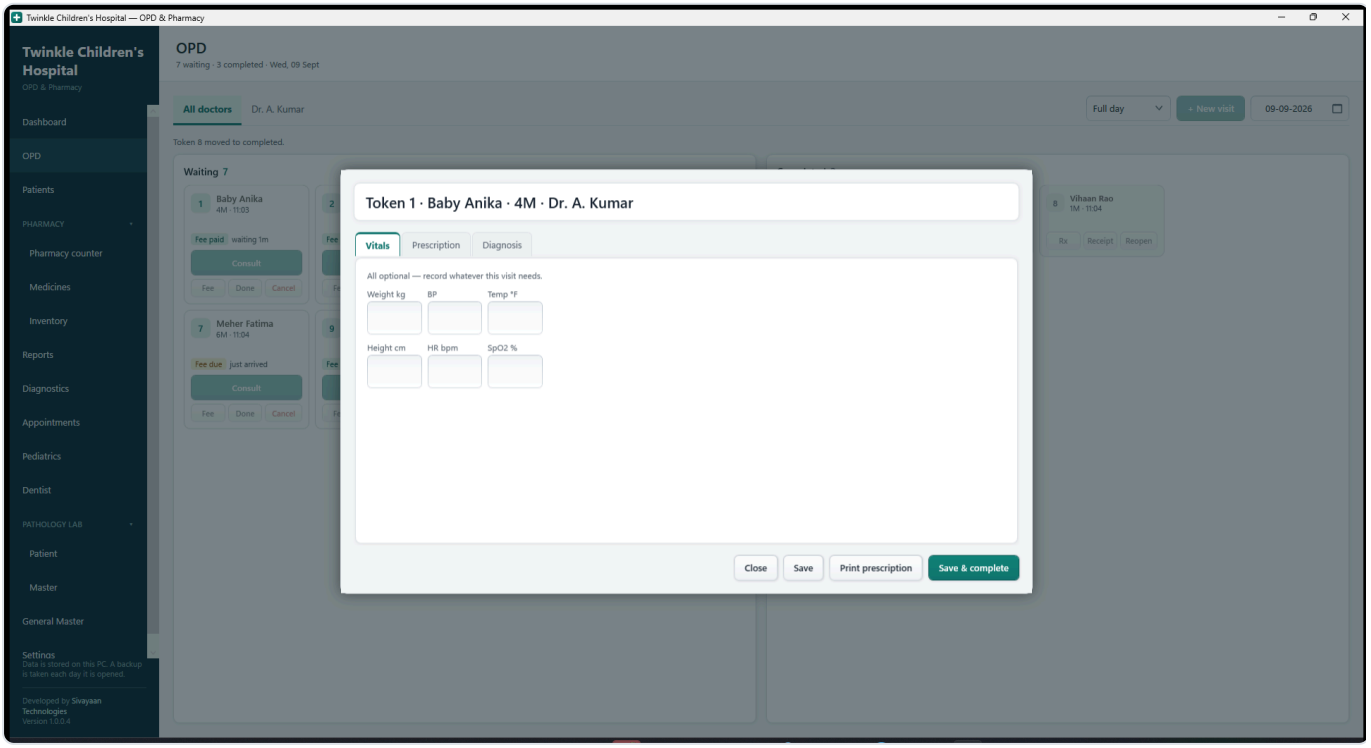
Afterwards the badge on the tile turns green to **Fee paid**, and the message under the doctor tabs gives the receipt number. Pressing **Fee** again does nothing; use **Receipt** on the completed tile, or the patient's own record, to print it a second time.

> **Why it asks at all.** A receipt is numbered and dated the moment it is written. There is no way to un-write one — a fee taken wrongly has to be reversed on paper. Pressing Fee used to take the money immediately, at whatever payment method a box at the top of the screen had been left on, and go straight to a print preview. Now nothing happens until you have seen the amount, the method and the name.

---

6. The consultation

Click **Consult** on a tile. Three tabs: **Vitals**, **Prescription**, **Diagnosis**.



The heading shows the token, the patient, their age and sex, and the doctor.

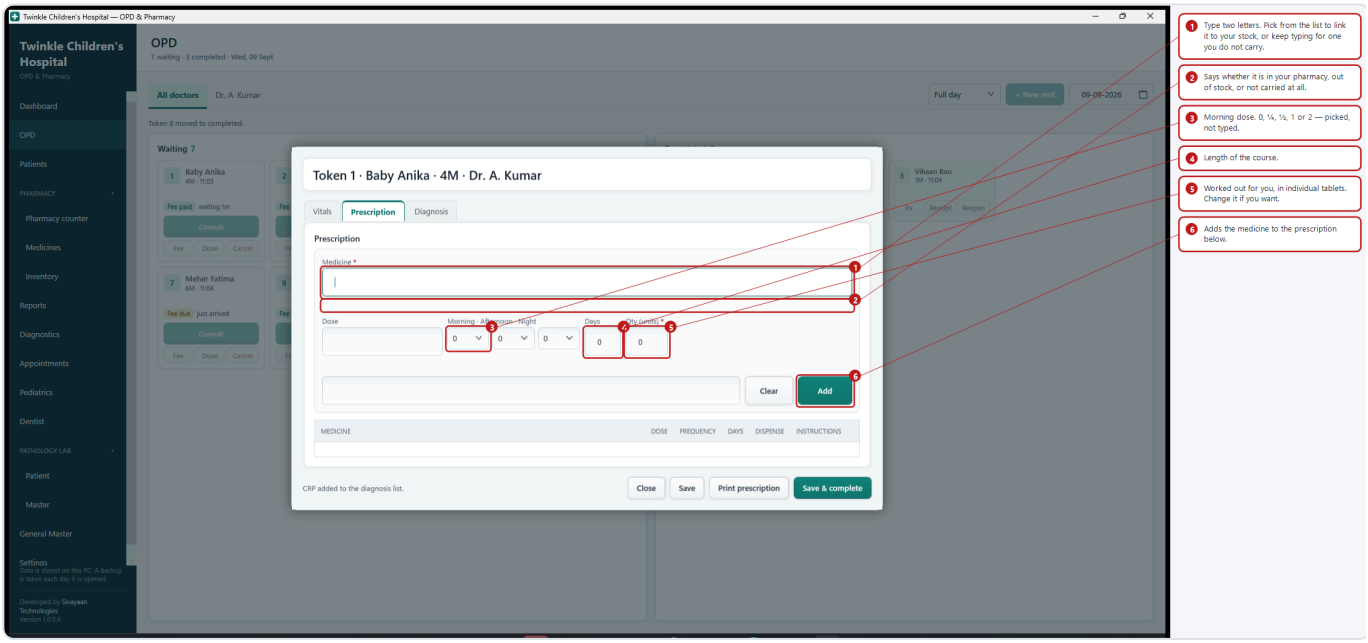
Vitals

Field	Notes
Weight kg, **BP**, **Temp °F**	
Height cm, **HR bpm**, **SpO2 %**	

All six are optional. Record whatever this visit needs.

Prescription

Every control, ringed and explained:



Fill the form at the top and press **Add**. Each medicine then appears in the list below.

Control	Notes
Medicine	Type two letters or more to search our pharmacy. Matches appear underneath — click one to use it
Dose	e.g. `5 ml` , `1 tab`
Morning · Afternoon · Night	Three lists: `0` , `1/4` , `1/2` , `1` , `2` . Pick how many at each time of day
Days	Length of the course
	Nothing is filled in for you — a dose is the doctor's decision, not the software's
Qty (units)	**Individual tablets.** Worked out for you — change it if you want
Grey line under the fields	What the course comes to, in tablets and in strips

Control	Notes
Instructions	e.g. "after food". Printed under the line
Add	Adds the medicine to the list
✕ in the list	Removes that medicine

The three lists replace the old typed frequency box. `1-0-1` was quick for anyone who already knew the notation and a guess for everyone else; picking from three lists cannot be mistyped, and the prescription still prints in the familiar `1-0-1` form.

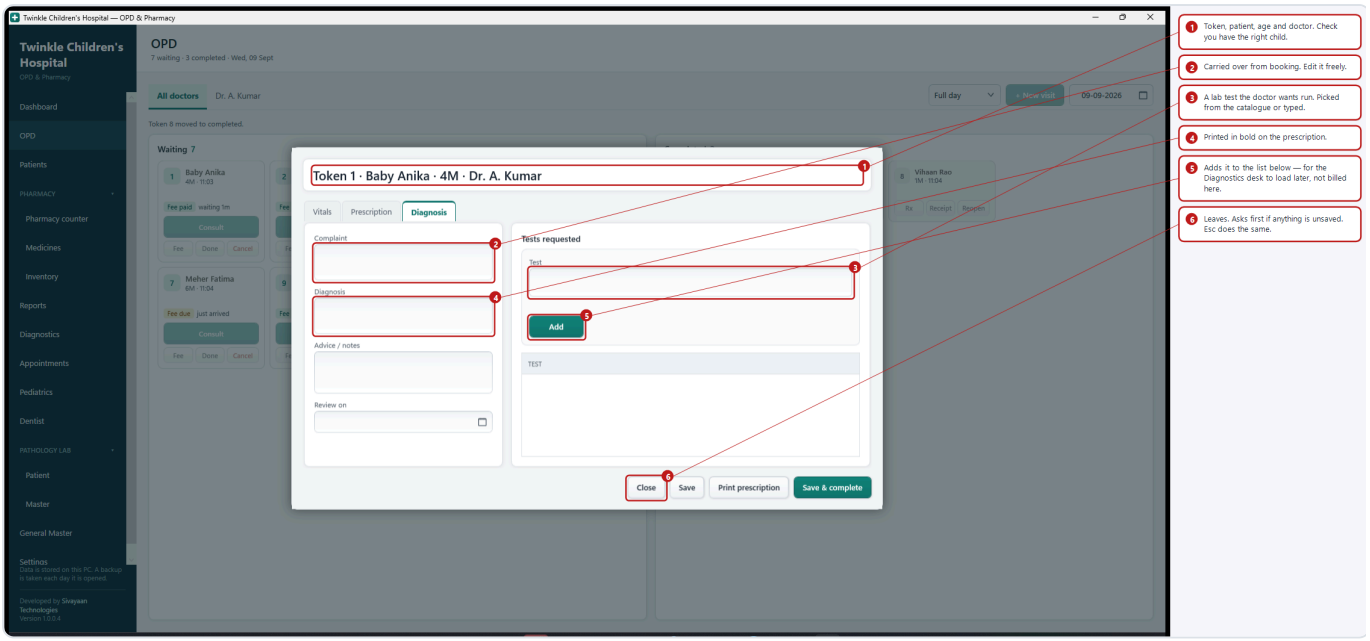
Add empties the whole row — dose, frequency, days and quantity included — not just the medicine name. A dose left behind from the last medicine would read as chosen for this one, and a wrong dose nobody typed is worse than retyping a right one.

> ### Quantity is always in individual units
>
> You pick `1` , `0` , `1` for `3` days and it fills in **6** — six tablets, not six strips. The line underneath shows what the pharmacy will hand over, e.g. "*6 units · 1 × 10 TAB minus 4*", so there is never any doubt about whether a number means tablets or strips.
>
> `SOS` and `PRN` have no fixed daily dose, so nothing is filled in and you type the quantity yourself.

> ### Prescribing something the clinic does not stock
>
> Type the name and simply **do not pick anything from the list**. The line reads "*Not in our pharmacy — it will be written on the prescription only*", and it is added exactly as typed. The parent buys it from an outside chemist.
>
> **It is never added to our medicine records.** A name typed on a prescription does not create a medicine, a batch, or a price — our catalogue only ever grows when stock is received.
>
> A medicine you **do** pick from the list shows "*In our pharmacy · 60 in stock*" and can be pulled straight onto a bill at the counter, with the quantity already correct. One prescription can mix the two freely.

Diagnosis

Every control, ringed and explained:



Field	Notes
Complaint	Carried over from booking; edit freely
Diagnosis	Printed in bold on the prescription
Advice / notes	Printed under the medicines
Review on	Follow-up date. Printed as "Review on 01 Aug 2026"
Tests requested	Only shown once the **Diagnostics** module is switched on — section 11

Tests requested works like the prescription: type two letters to search the test catalogue, click a match or keep typing for one not in it, and click **Add**. Nothing is billed here — the list is picked up later at the Diagnostics desk with **Load diagnostic tests**, the same way a prescription is picked up at the pharmacy counter.

Fee is no longer set here — it is taken from the tile's own **Fee** button, section 5.

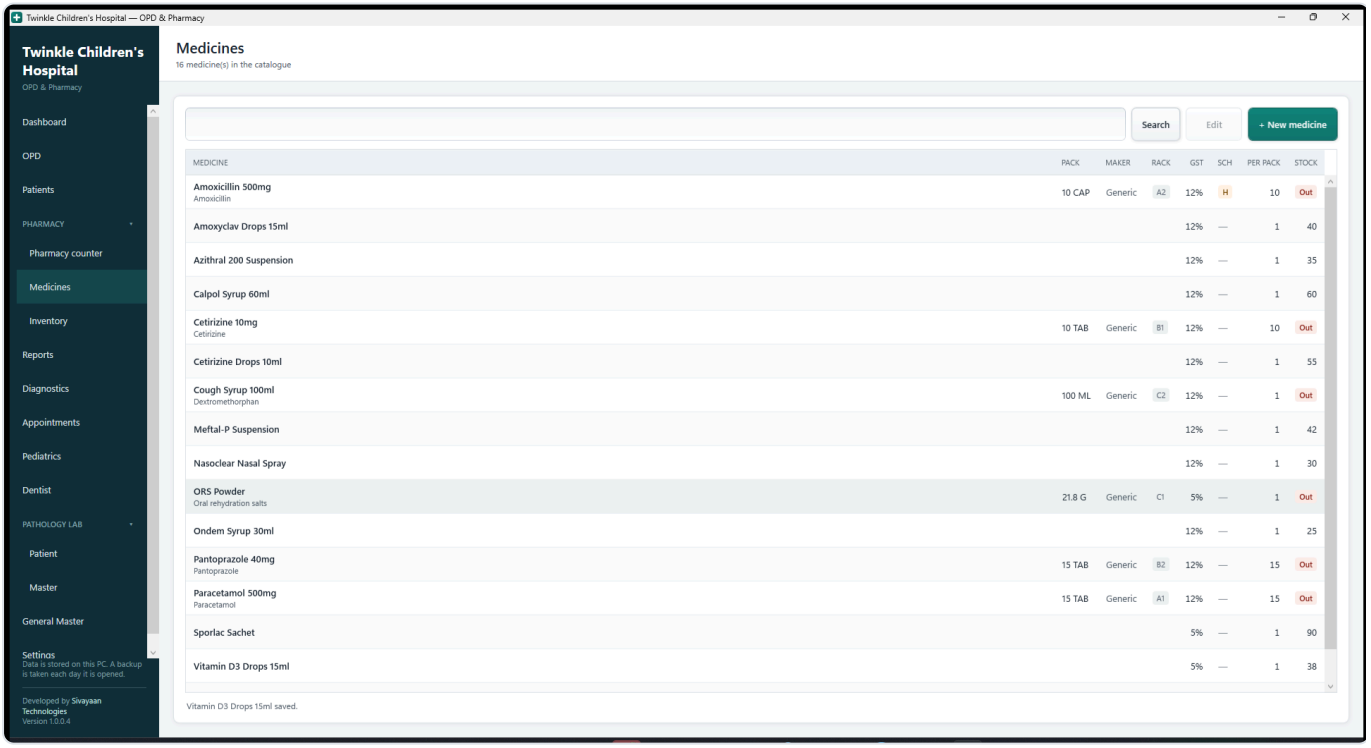
Bottom buttons

Button	What it does
Close	Leaves the consultation. Asks first if anything is unsaved
Save	Keeps everything. Patient stays in Waiting
Print prescription	Saves, then opens the print preview
Save & complete	Saves and **moves the tile to Completed**

The **Esc** key does the same as **Close**. While the consultation is open the rest of the app is greyed out and waits — you cannot wander off to another screen with a half-written prescription behind you.

7. Medicines — the catalogue

What each medicine **is**. Set up once per medicine; stock is the next screen.



The catalogue

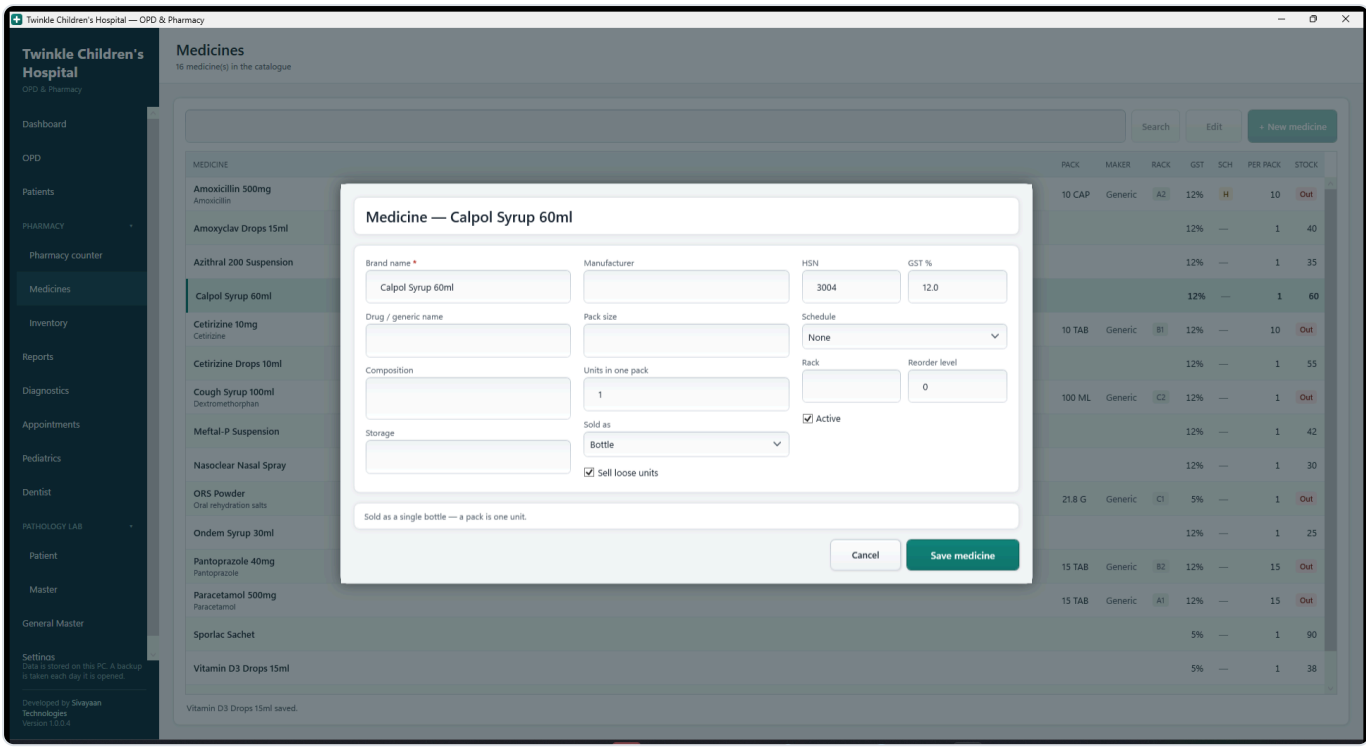
The grid has the whole window: medicine, pack, maker, rack, GST %, schedule, units per pack and stock on hand. Above it sit the search box, **Search**, **Edit** and **+ New medicine**.

Search matches the **brand name**, the **drug name**, the manufacturer and the rack. Typing `paracetamol` finds Calpol; typing `calpol` finds it too.

To change a medicine, select the row and click **Edit** — or just **double-click the row**, which does the same thing.

The medicine form

Opens over the screen, in three columns: what it is called, what a pack is, and what it costs and where it lives.



Field	Notes
Brand name	What is printed on the pack — *Calpol*. The only required field
Drug / generic name	The drug itself — *Paracetamol 250mg*. Searched as well
Manufacturer	Company name
Pack size	As printed, e.g. `10 TAB`, `60ML`
HSN	Tax code. `3004` covers most formulations
GST %	Usually 5 or 12
Schedule	`None`, `H`, `H1`, `X`. H1 sales are recorded in a register
Rack	Where it sits on the shelf. Searchable
Reorder level	Below this it appears in the Low stock report
Units per pack	**10 for a ten-tablet strip. 1 for a syrup bottle**
Sell loose units	Ticked, part of a strip can be sold. Unticked, the counter insists on whole packs
Active	Uncheck to hide it from the counter
Save medicine	Saves it and closes the form. The catalogue clears — search box included — for the next medicine
Cancel	Closes without saving. Nothing already saved is changed

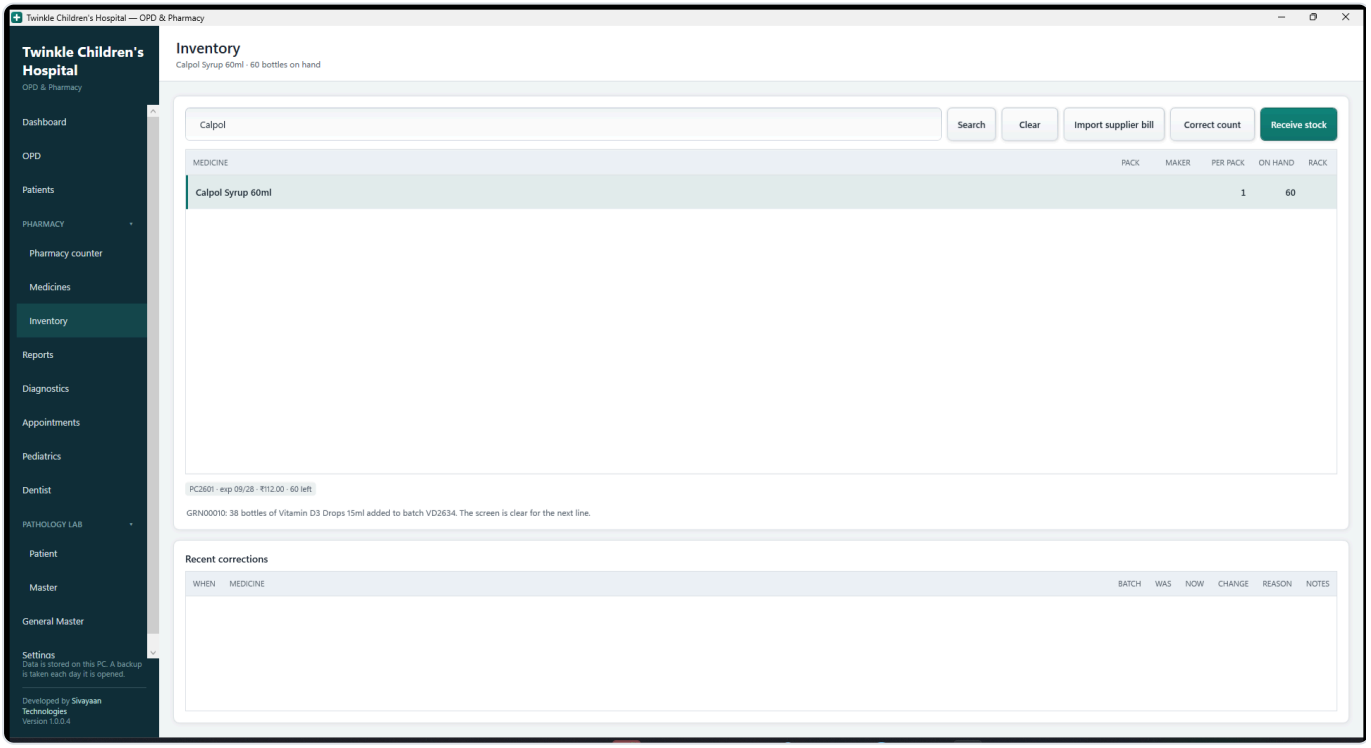
There is no Clear button, because there is nothing to clear: the form holds one medicine and Cancel takes it away. The next one gets a fresh form.

> **Units per pack is what makes loose sale work**
>
> Set it to `10` for a strip of ten tablets and stock is counted in **tablets**, so a customer can buy five. Leave it at `1` for a syrup bottle — half a bottle is not a thing you can sell.

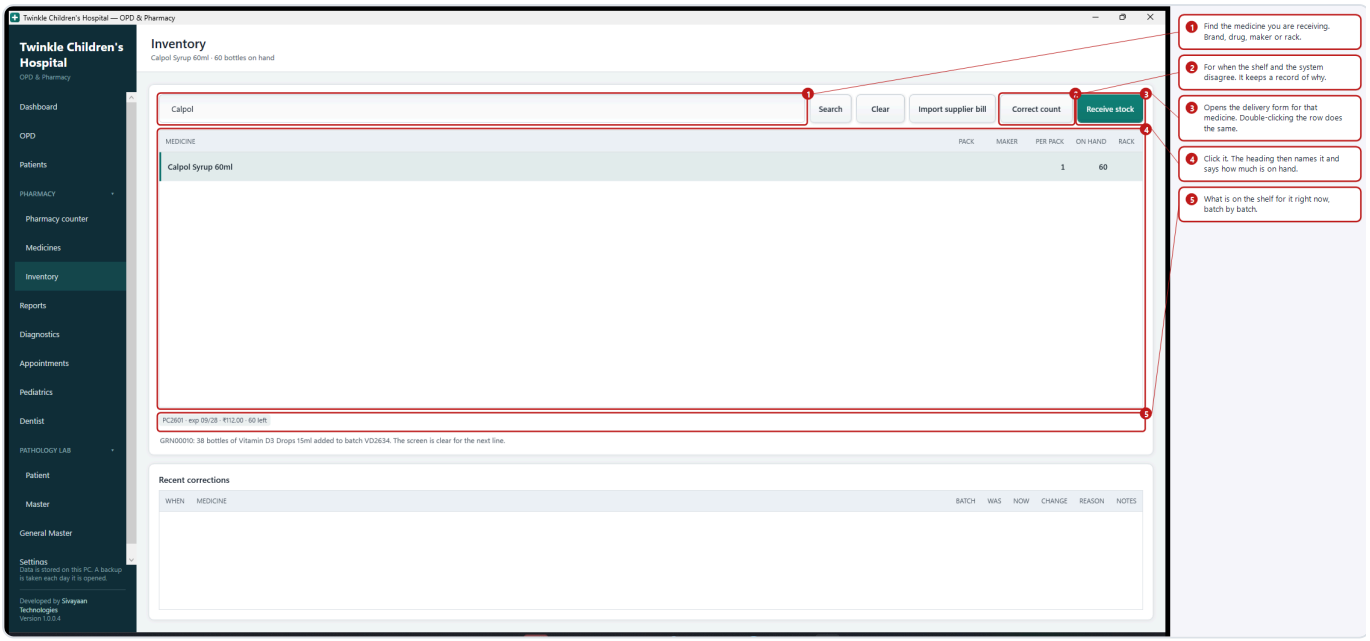
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8. Inventory — what is on the shelf

Everything to do with stock: receiving it, seeing the batches, correcting a count. Nothing here changes what a medicine *is* — that is the Medicines screen.



Every control, ringed and explained:



Finding the medicine

Type any part of the brand name, drug name, maker or rack and click **Search**, then click the row. The heading at the top of the screen then names the medicine and how much is on hand, so you always know what you are about to change.

Underneath the grid, **the batches on the shelf** for the selected medicine appear as small chips — batch number, expiry, MRP and how much is left.

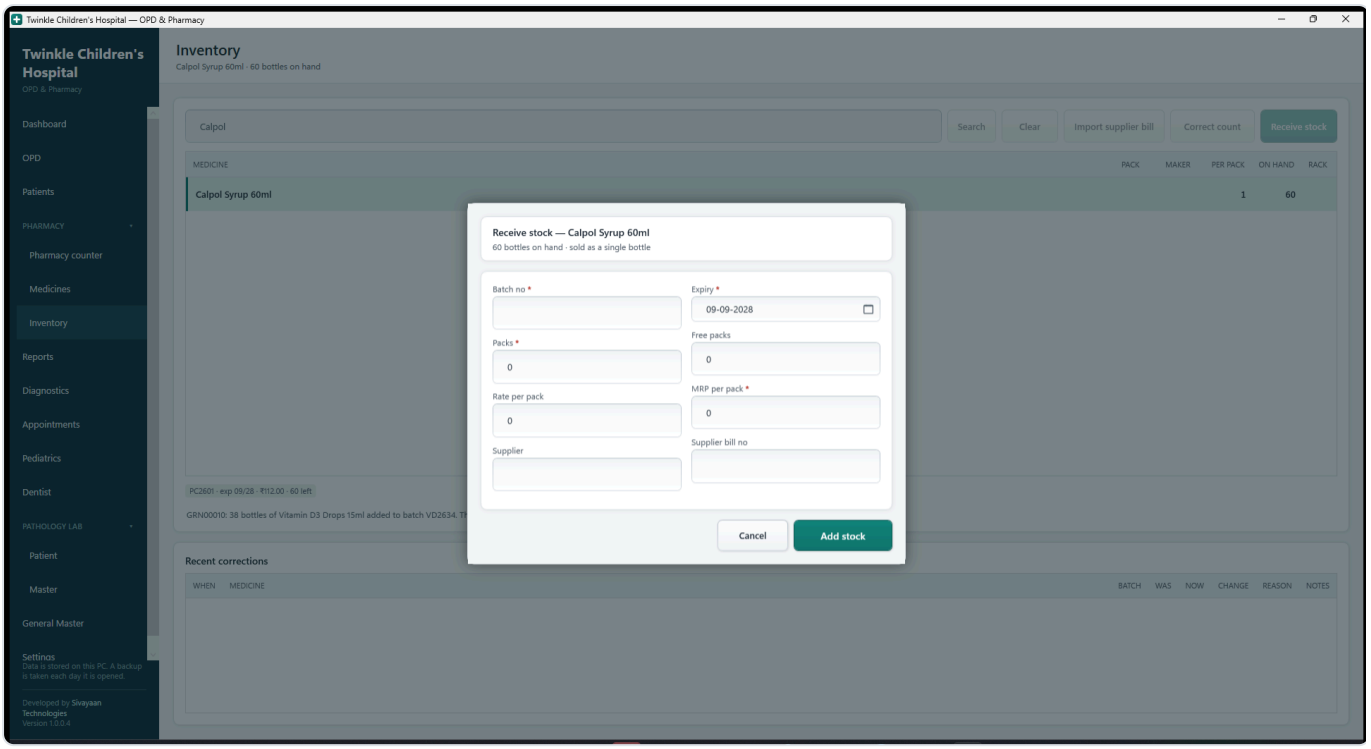
Along the top: **Search**, **Clear**, **Import supplier bill** (see section 9), **Correct count** and **Receive stock**. The last two need a medicine selected first.

At the foot of the screen, **Recent corrections** lists every count that has been put right — when, what, was, now, the change, the reason and any note.

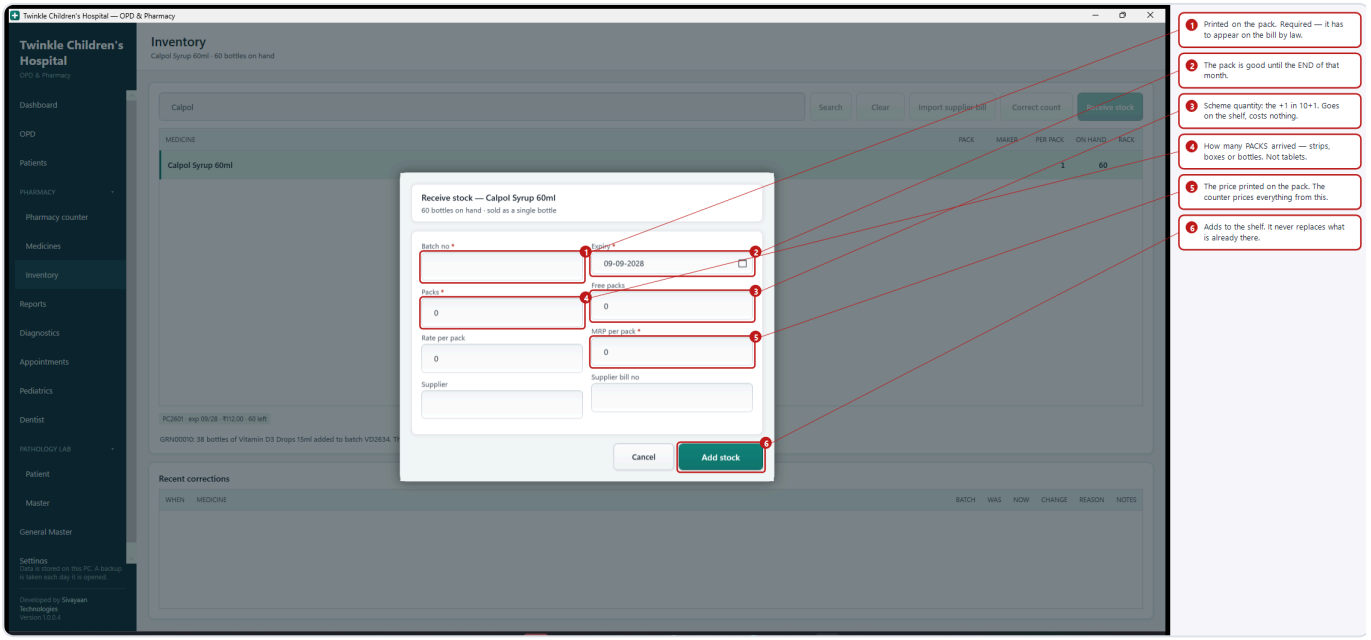
Receive stock

Select the medicine, then click **Receive stock**. A form opens over the screen, headed with the medicine's name and what is already on the shelf.

This is the only way stock enters, and it always creates a batch.



Every control, ringed and explained:



Field	Notes
Batch no	Printed on the pack. **Required — it goes on the bill by law**
Expiry	The pack is good until the **end** of that month
Packs	How many **packs** arrived — strips, boxes, bottles
Free packs	Scheme quantity, the "+1" in 10+1. Adds to stock, costs nothing
Rate per pack	What the hospital paid per pack

Field	Notes
MRP per pack	The price printed on the pack. **The counter prices from this**
Supplier, **Supplier bill no**	For your records, and what the reconciliation report matches against
Add stock	Adds it and closes the form, ready for the next line
Cancel	Closes it. Nothing goes on the shelf

> ### Packs in, units out
>
> Above the buttons a grey line reads back what you have entered: "*20 pack(s) × 15 = 300 tablets onto the shelf*". You count deliveries in strips, the counter sells tablets, and this is the one place the two meet — so it says so out loud rather than leaving you to trust it.

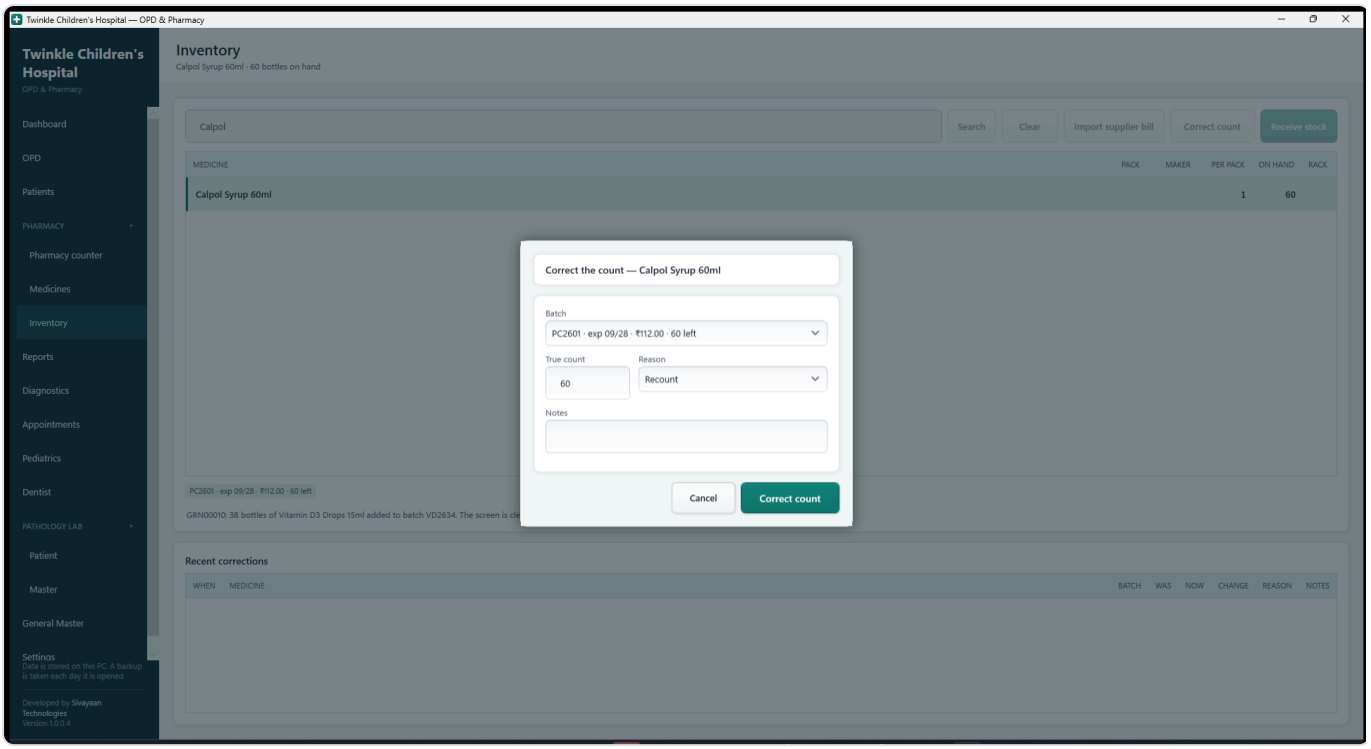
Everything goes when the form closes — the supplier and bill number included — so a price, an expiry or a supplier from the last delivery can never be carried onto the wrong batch, and a second batch can never be received against a medicine you had forgotten was still selected.

Double-clicking a row in the grid opens this form too.

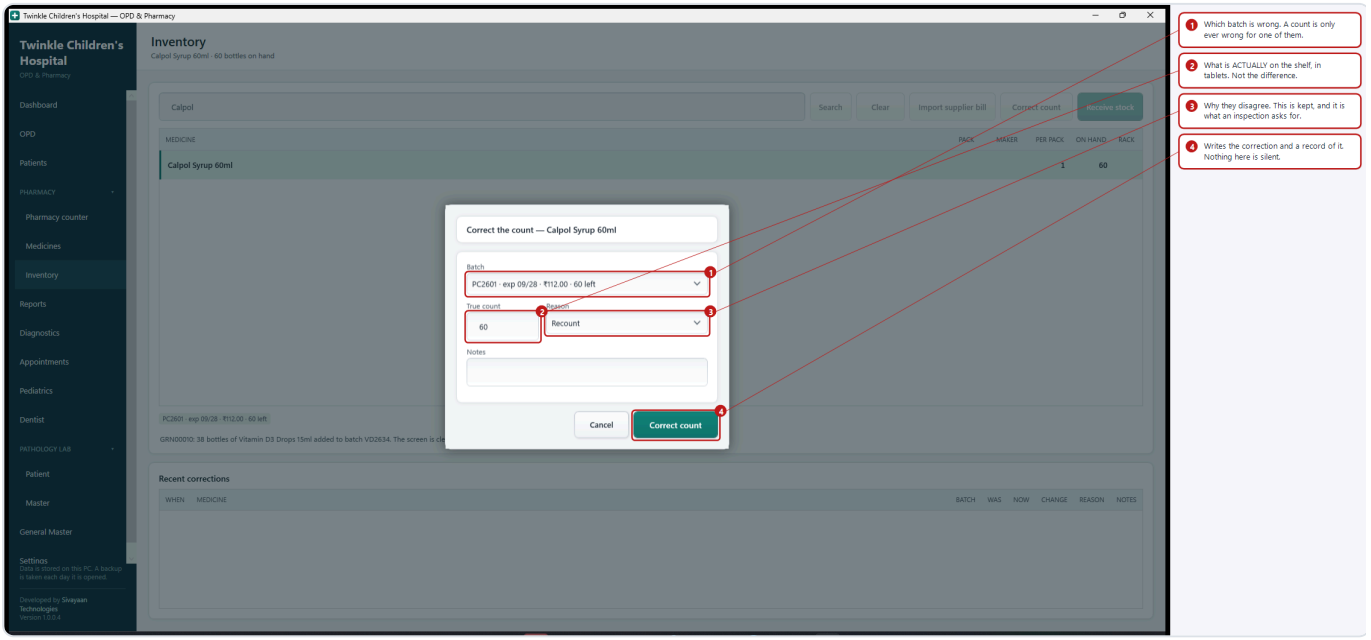
> **Adding stock always adds.** Receiving the same batch number again increases what is on the shelf. It never replaces it.

Correct count

For when the shelf and the screen disagree — breakage, a miscount, or something keyed in wrongly. Select the medicine and click **Correct count**.



Every control, ringed and explained:



Control	Notes
Batch	Which batch is wrong. Shows expiry, MRP and current count. Chosen for you when there is only one
True count	What is **actually** on the shelf, in units. Starts at what the system believes
Reason	`Recount` , `Breakage` , `Expired` , `Lost` , `Entry error` , `Other`
Notes	Free text — worth writing for anything unusual
Correct count	Applies it and closes the form

Control	Notes
Cancel	Closes it. Nothing is corrected

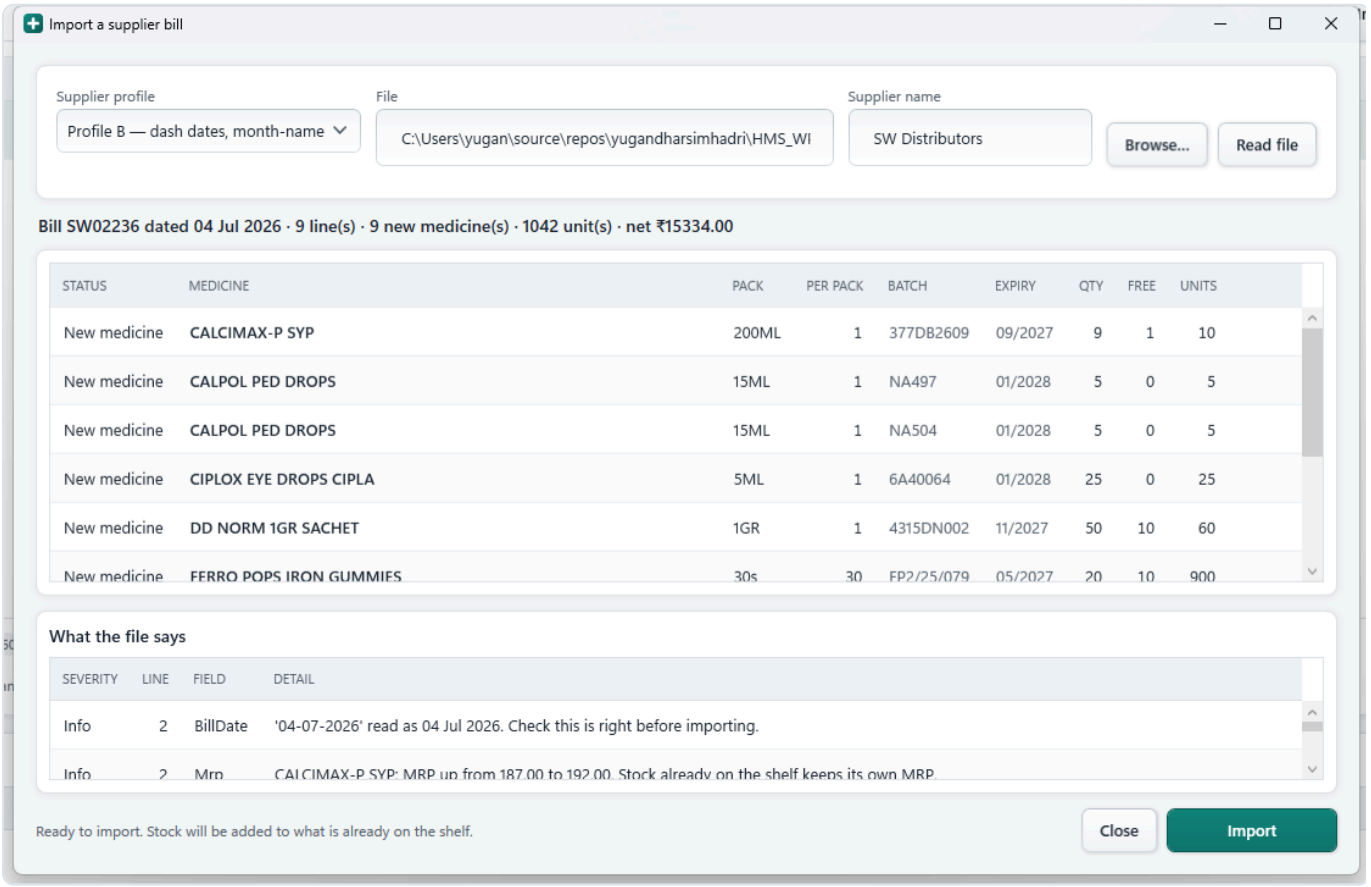
If the medicine has nothing on the shelf, the form does not open at all — it says so instead. There is no count to put right, and an empty batch list only invites a correction against whatever else was selected.

> **Every correction is recorded**
>
> Stock otherwise only moves by receiving or selling, and both leave a document. A manual correction has none, so it writes its own — otherwise a shortfall looks the same as theft and nobody can say what happened.
>
> The correction and its record are saved together: you cannot get one without the other. Setting the count to what it already is is refused, and stock cannot go below zero.

---

9. Importing a supplier bill

Instead of keying in a delivery line by line, load the file your supplier sends. **Inventory → Import supplier bill.**



Step 1 — choose the profile and the file

Control	What it does
Supplier profile	Which supplier's format this is. Each knows that supplier's date and expiry style
File	The path. Use Browse
Supplier name	**Type this** — the file does not contain it
Browse…	Pick the file. Reads it immediately
Read file	Re-reads it after changing the profile

Step 2 — check what it will do

The summary line reads, for example: "*Bill SW02236 dated 04 Jul 2026 · 9 line(s) · 9 new medicine(s) · 1042 unit(s) · net ₹15334.00*"

Column	Meaning
STATUS	`Matched` — found in your catalogue · `Check` — a likely match, confirm it · `New medicine` — will be created
MEDICINE, **PACK**, **BATCH**, **EXPIRY**	As the supplier sent them
PER PACK	**Editable.** Units in one pack
QTY, **FREE**	Packs billed, and free packs
UNITS	What will land on the shelf: (qty + free) × per pack
RATE, **MRP**, **GST**	Cost, printed price, tax rate

What the file says below lists everything worth knowing — the bill date it read, MRP changes, anything it could not understand.

> ### Check the PER PACK column
>
> The file rarely states how many are in a pack. `30s` is read as 30 gummies. `60ML` is one bottle and stays `1` — a syrup is not sixty sellable units.
>
> For strips, **type the real number** before importing. Correct it once and the medicine remembers it.

Step 3 — import

Import writes everything in one go. **Close** abandons it — nothing is written until you click Import.

Afterwards it reports: "*Imported as GRN00003: 9 line(s), 9 new medicine(s), 1042 unit(s) added to stock.*"

What the import guarantees

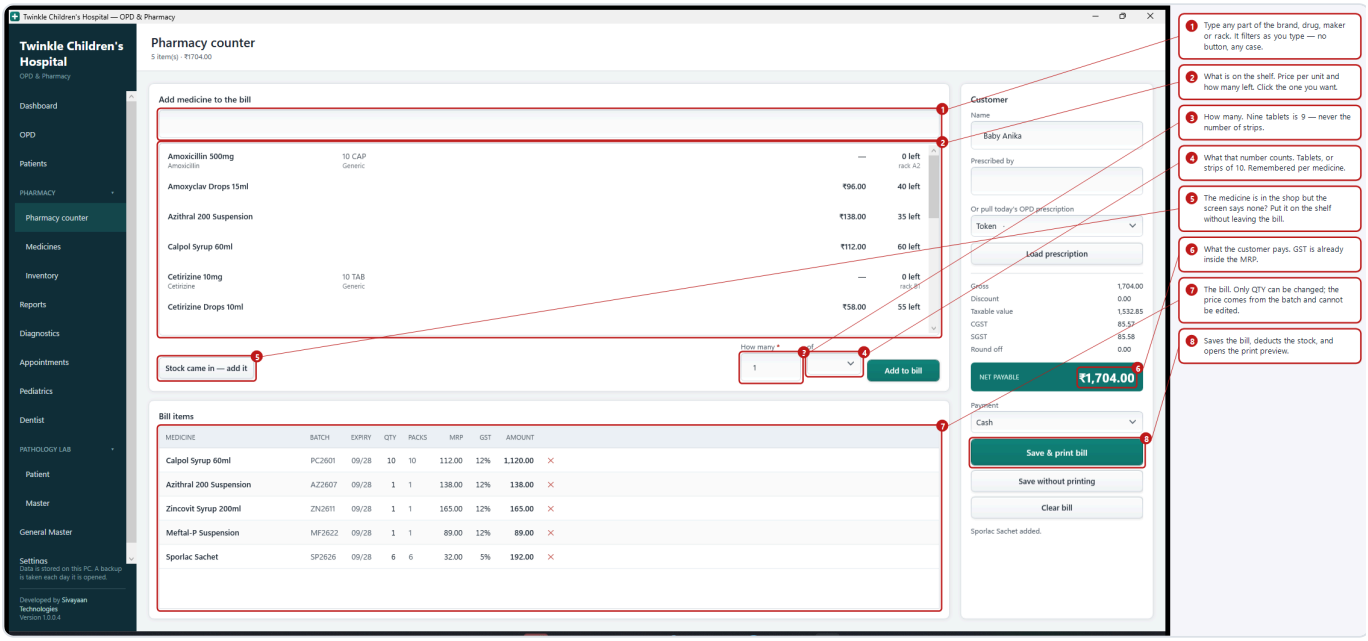
- **Stock is added**, never replaced. 12 counted by hand plus 5 on the bill is 17
- **The same bill cannot be imported twice.** It is refused by bill number
- **Nothing is written if anything fails.** All of it, or none
- **The supplier's product codes are remembered**, so their next bill matches by itself

---

10. The pharmacy counter

Three steps per line: **find the medicine, set the quantity, add.**

Every control, ringed and explained:



	Control	What it is for
1	**Search**	Any part of the brand, drug, maker or rack. Filters as you type, in any case
2	**Results**	What is on the shelf: price per unit, how many left, which rack
3	**How many**	The number. Nine tablets is `9`
4	**of**	What that number counts — *tablets* or *strips of 10*. Remembered per medicine
5	**Stock came in — add it**	Puts stock on the shelf without leaving the bill
6	**Bill items**	Only QTY can be changed. The price comes from the batch
7	**Net payable**	What the customer pays. GST is already inside the MRP
8	**Save & print bill**	Saves, deducts the stock, opens the preview

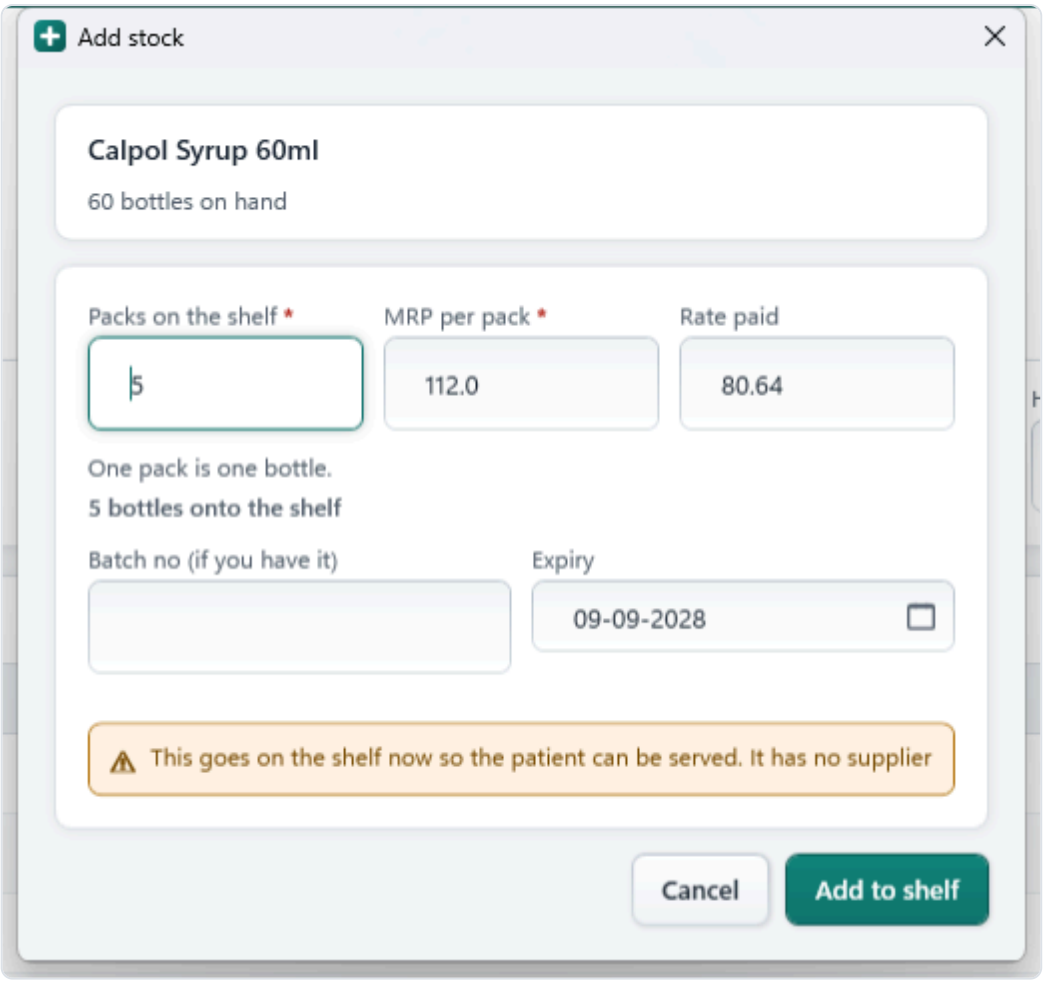
Add medicine to the bill

Control	What it does
Medicine	Type part of the name. Enter searches
Find	Runs the search
Results list	Shows pack, maker, rack and stock. Click one
Batch	**The nearest-expiry batch with stock is chosen for you**
Qty (units)	**Tablets, not strips.** `5` sells five out of a strip
Add to bill	Adds the line, then empties the search and lets go of the medicine. The bill is untouched — type the next medicine's name to carry on
Stock came in — add it	Puts stock on the shelf from here. See below

The medicine is in the shop but the screen says none

It happens: a delivery arrived and nobody entered it, or it was bought in as a one-off. **Do not send the patient away and do not leave the bill.**

Click **Stock came in — add it**.



Field	Notes
Packs on the shelf	How many strips, boxes or bottles you are putting in
MRP per pack	**Required** — nothing can be priced without it. Filled in from the last time this medicine came in
Rate paid	Leave it if you do not know. It can be filled in when the bill arrives
Batch no	Enter it if it is on the pack. **Leave it blank and one is allocated for you**, starting `CTR-`
Expiry	Defaults to two years out. Change it if you know better
Add to shelf	Puts it in. You come straight back to the bill

The grey line reads back what you have entered — "*5 pack(s) × 10 = 50 tablets onto the shelf*" — before anything is committed.

> ### What this costs you, and why it is worth it
>
> Stock added this way has **no supplier bill behind it**, so total purchases will not tie out against total sales until that bill turns up. That is a deliberate trade: a counter that stops to do paperwork with a patient waiting is a counter nobody uses.
>
> Nothing is hidden to buy that. Every entry is a proper goods-inward document with the quantity, the MRP, the date and who keyed it, and every one of them is listed under **Reports → Stock to reconcile** until it is matched to the real bill. You can square the books whenever you like — a week later, a month later — because nothing was done quietly.

Bill items

Column	Notes
MEDICINE, **BATCH**, **EXPIRY**	What is being handed over
QTY	Base units. Editable
PACKS	Reads back as `2 × 10 TAB + 3`
MRP, **DISC %**	Editable
GST %, **AMOUNT**	Calculated
×	Removes the line

Customer (right)

Control	Notes
Name	Defaults to `Guest`. **A walk-in needs no patient record**
Prescribed by	Doctor's name. Required on the bill for a Schedule H1 drug
Or pull today's OPD prescription	Pick a patient seen today
Load prescription	Puts every prescribed medicine that is in stock on the bill

Totals

```
Gross            what the MRP comes to
Discount         anything taken off
Taxable value    the MRP with GST taken back out of it
CGST + SGST      the tax, split in half
Round off        to the nearest rupee
NET PAYABLE      what the customer hands over
```

Control	What it does
Payment	Cash, UPI or Card
Transaction / reference no.	Only shown for UPI or Card. Never required — for reconciling against the gateway or bank statement
Save & print bill	Saves and opens the preview
Save without printing	Saves only
Clear bill	Empties the counter without saving

Every bill is settled in full

There is no credit and no part payment — by design, because the clinic does not offer them. A bill is paid before it is saved, so nothing is ever outstanding and there is no balance to chase.

MRP already includes GST

Tax is never added on top. Ten strips at ₹112 MRP come to **exactly ₹1,120** — of which ₹1,000 is the taxable value and ₹120 is GST.

A whole strip always costs what is printed on it

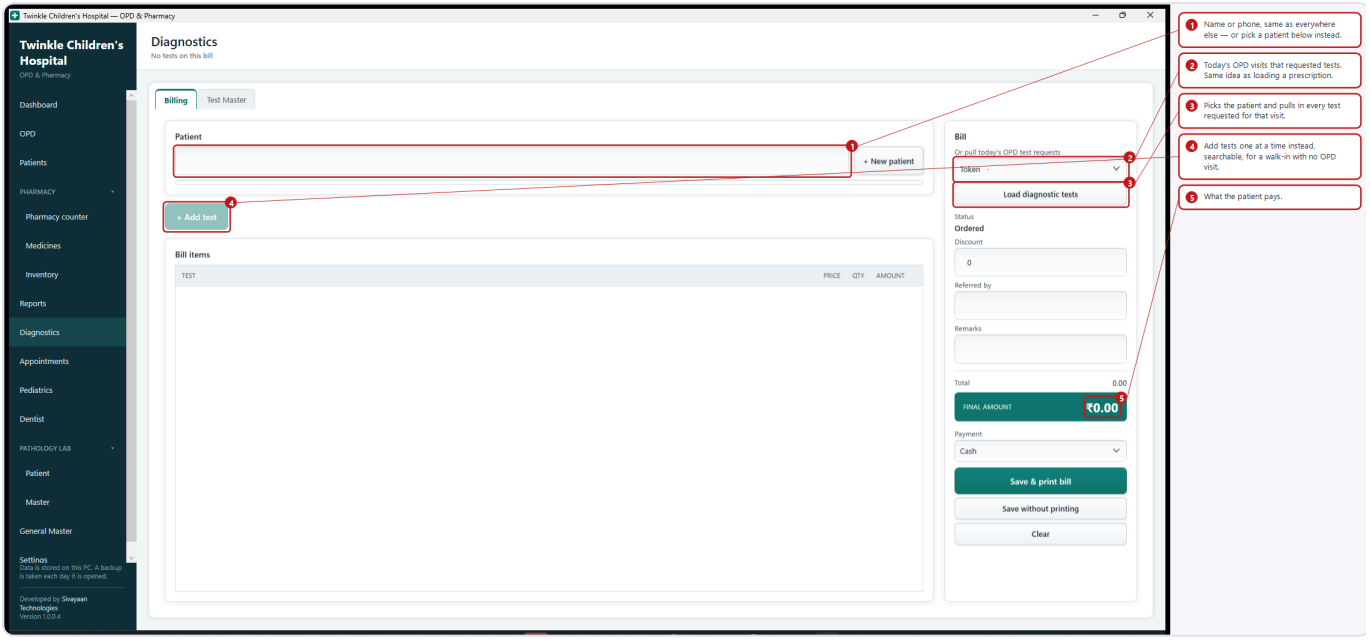
Five tablets from a ₹112 strip of ten cost ₹56. But a **full** strip costs ₹112, not ten times the rounded per-tablet price. This matters where the price does not divide evenly: ₹87.50 across fifteen tablets is ₹5.83 each, and the full strip still costs ₹87.50.

11. Diagnostics

Optional — off by default. Switched on from **Settings** → **Features**, and once on it stays on; the nav button appears immediately, no restart needed. Two tabs: **Billing** and **Test Master**.

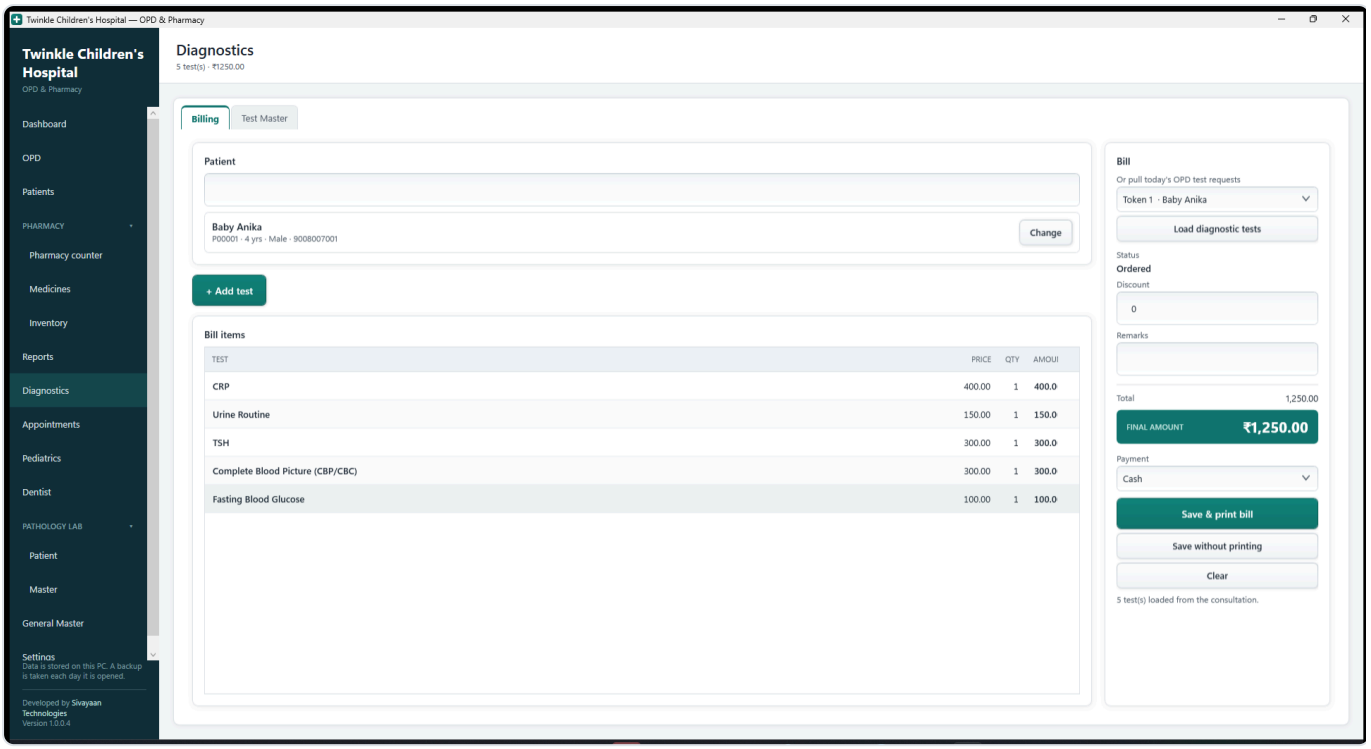
Billing

Every control, ringed and explained:



	Control	What it is for
1	**Patient**	Search by name or phone, same as everywhere else
2	**Or pull today's OPD test requests**	Today's OPD visits that requested tests and are not yet billed
3	**Load diagnostic tests**	Picks the patient from that visit and pulls in every test requested for it
4	**+ Add test**	A searchable popup listing every active test — tick as many as needed. Only enabled once a patient is chosen
5	**Final amount**	What the patient pays

With a patient chosen and at least one test on the bill:



Field	Notes
Status	`Ordered` until first saved — shown as plain text, since there is nothing yet to move it along. A saved bill gets a real dropdown: `Ordered → Sample Collected → Result Received → Completed`
Discount	
Referred by	**Only shown for a patient who did not come through this clinic's own OPD** — one loaded via **Load diagnostic tests** is referred by the clinic itself, so there is nothing to ask
Remarks	
Payment	Cash, UPI or Card
Transaction / reference no.	Only shown for UPI or Card. Never required
Save & print bill / **Save without printing** / **Clear**	

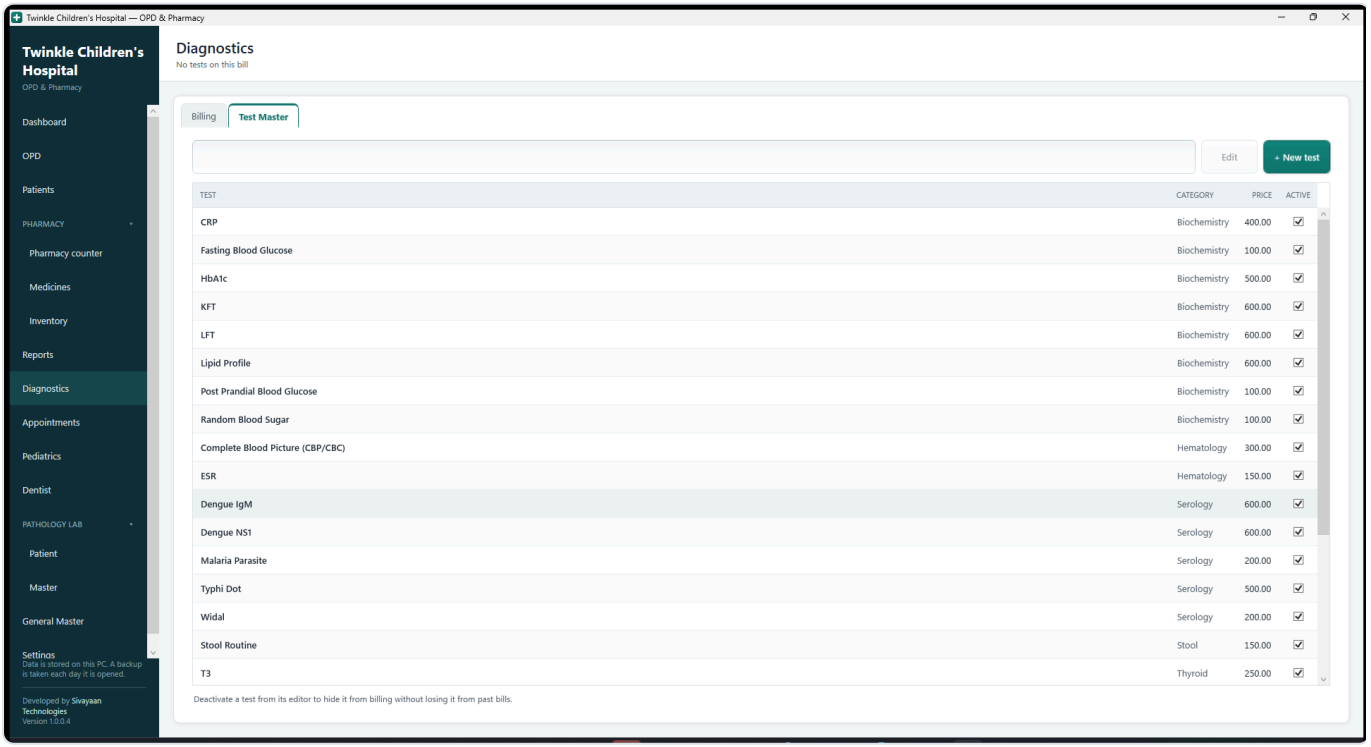
A bill **Completed** can no longer be edited — the fields grey out. Move it along the status list only as far as it has actually got.

Two ways in, same result

+ Add test is for a walk-in with no OPD visit today — search the patient, then search and tick tests one at a time. **Load diagnostic tests** is for a patient the doctor has already seen — it does the patient and every requested test together in one click, the diagnostics equivalent of **Load prescription** at the pharmacy counter. Either can add more tests to the same bill afterwards.

Test Master

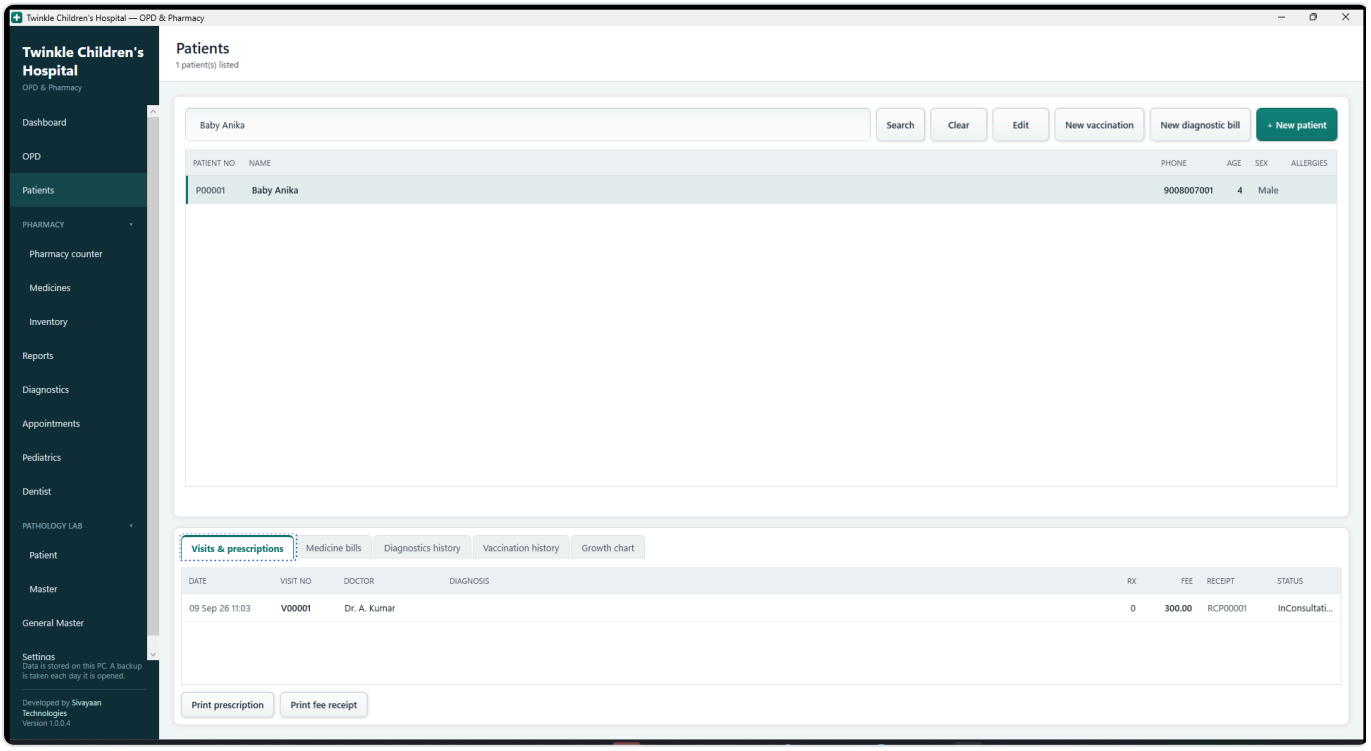
The list **+ Add test** and **Load diagnostic tests** both draw from.



Control	What it does
Search	By name or category
+ New test	Opens a popup: name, category, price
Edit	Opens the highlighted test. Double-clicking the row does the same
Active	Tick to show it in billing. Deactivate a test rather than delete it once it has been billed — past bills keep their own price regardless

A set of common tests is preloaded on first run — complete blood picture, blood sugar variants, liver and kidney function, thyroid, dengue and malaria screens, urine and stool routine, vitamins, and more — editable from here. Nothing is ever re-inserted or overwritten on a later update.

12. Patients



Search by **name, patient number or phone**. A phone number lists the whole family.

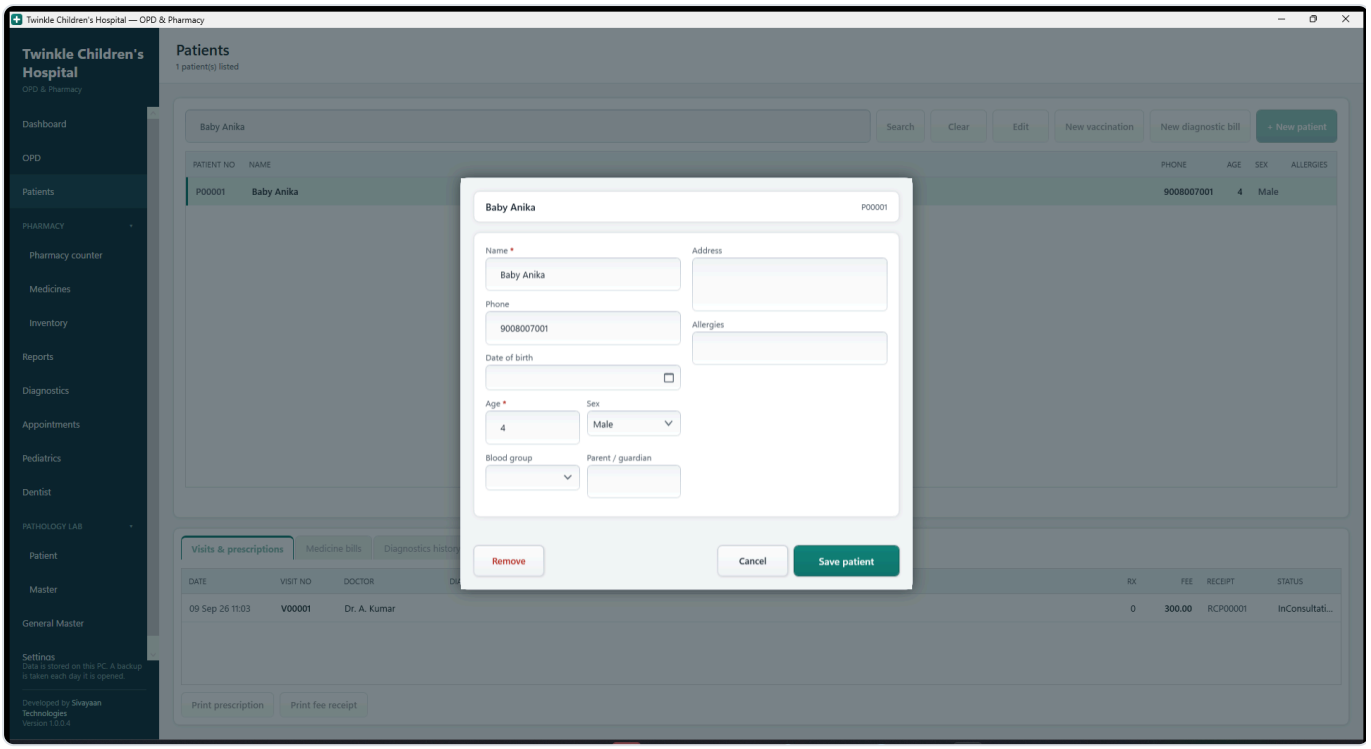
The register

Patient no, name, phone, age, sex and allergies, across the whole window. Click a row to select it — the history underneath fills in with that patient's visits and bills.

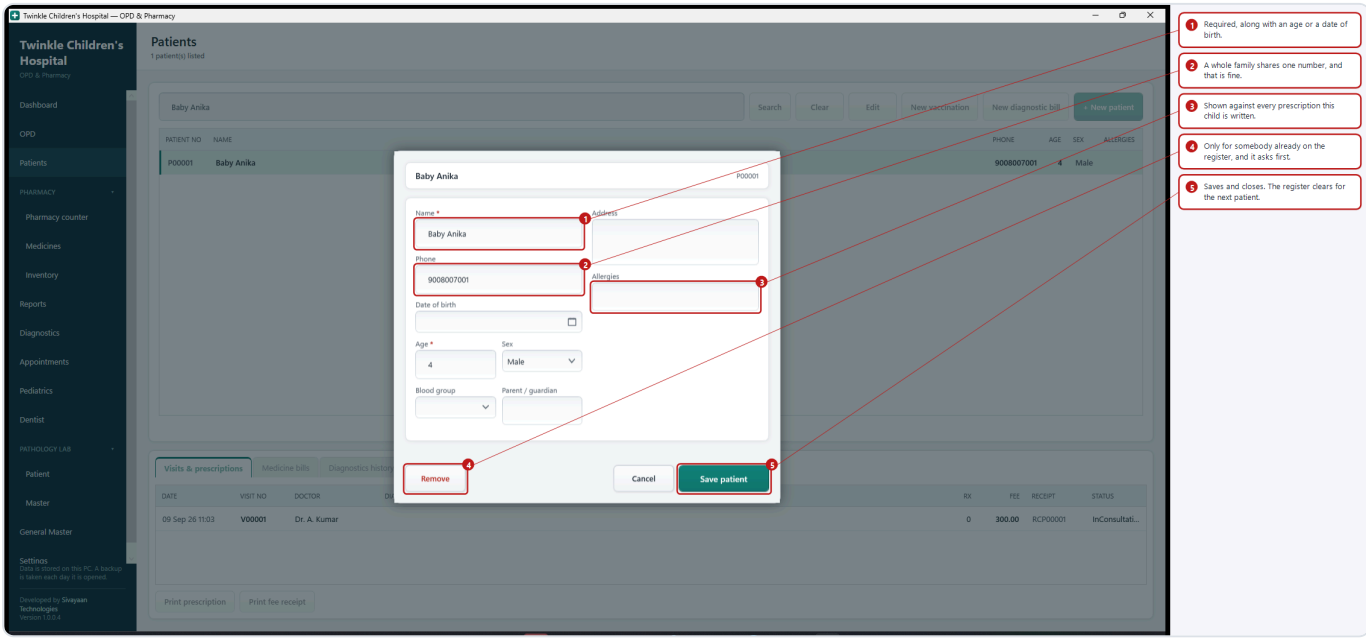
Along the top: **Search**, **Clear**, **Edit**, **New vaccination** and **New diagnostic bill** (each shown only once its own module is on — jumping straight to Pediatrics or Diagnostics with this patient already chosen), and **+ New patient**.

The patient form

+ New patient registers someone without booking a visit. **Edit** opens the selected patient — as does **double-clicking the row**.



Every control, ringed and explained:



Field	Notes
Patient no	Top right. Allocated on save, e.g. `P00012`
Name	Required
Phone	The parent's number. Shared across siblings is normal
Date of birth	Optional — set it and **Age fills in and locks itself**, worked out from it
Age — Years	Required **unless a date of birth is set**. Use **0** for a baby under one

Field	Notes
Age — Months	Optional, 0 to 11. Blank and **0** mean the same. Never enough on its own — a baby is **0 years** and some months. Locks along with Years when a date of birth is set
Sex	
Blood group	Optional
Parent / guardian	Optional, and only shown once the age is under 18
Address, **Allergies**	Optional. Allergies show against every prescription
Save patient	Saves and closes. The register clears — search box included — for the next patient
Cancel	Closes without saving. Nothing already saved is changed
Remove	Only for somebody already registered, and it asks first. Refused if they have visits on record

Name, and either a date of birth or the age in years, are required. Nothing else is. Ages print the way a clinic says them — **7m**, **4y 3m**, **41y** — on the register, the queue tiles, the receipt and the prescription.

> **The register clearing after Save is deliberate.** This screen used to keep the patient loaded in a column on the right, and typing the next child's name over it and pressing Save changed the child you had just registered instead of adding the new one — the first child left the register without a trace. Each patient now gets a form of its own. To carry on editing the same patient, search for them again and click Edit.

History

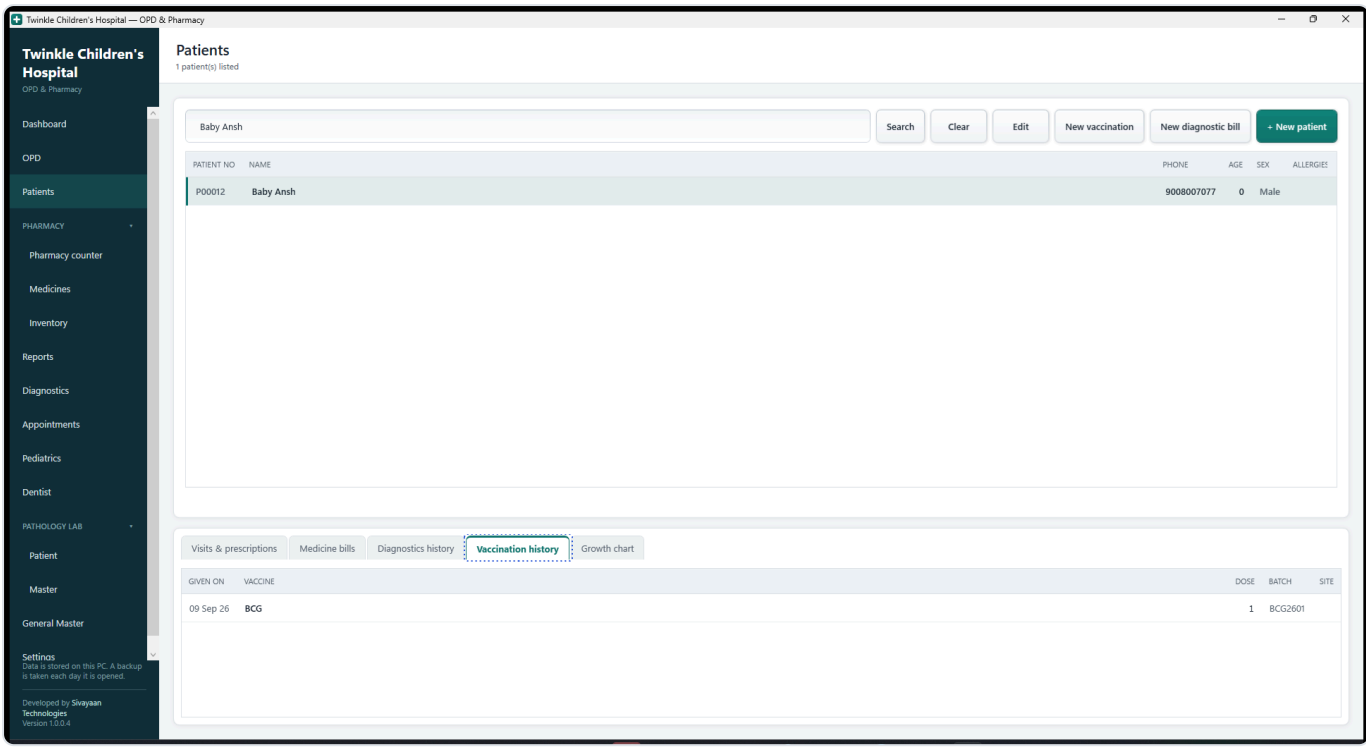
Visits & prescriptions — every visit ever, with diagnosis, fee and receipt number. Select one, then:

- **Print prescription** — prints it however long ago it was
- **Print fee receipt** — prints the receipt again, marked DUPLICATE

Medicine bills — every bill for this patient, with **Print bill**.

Diagnostics history — every diagnostic bill for this patient, with its status and **Print bill**. Only shown once that module is on.

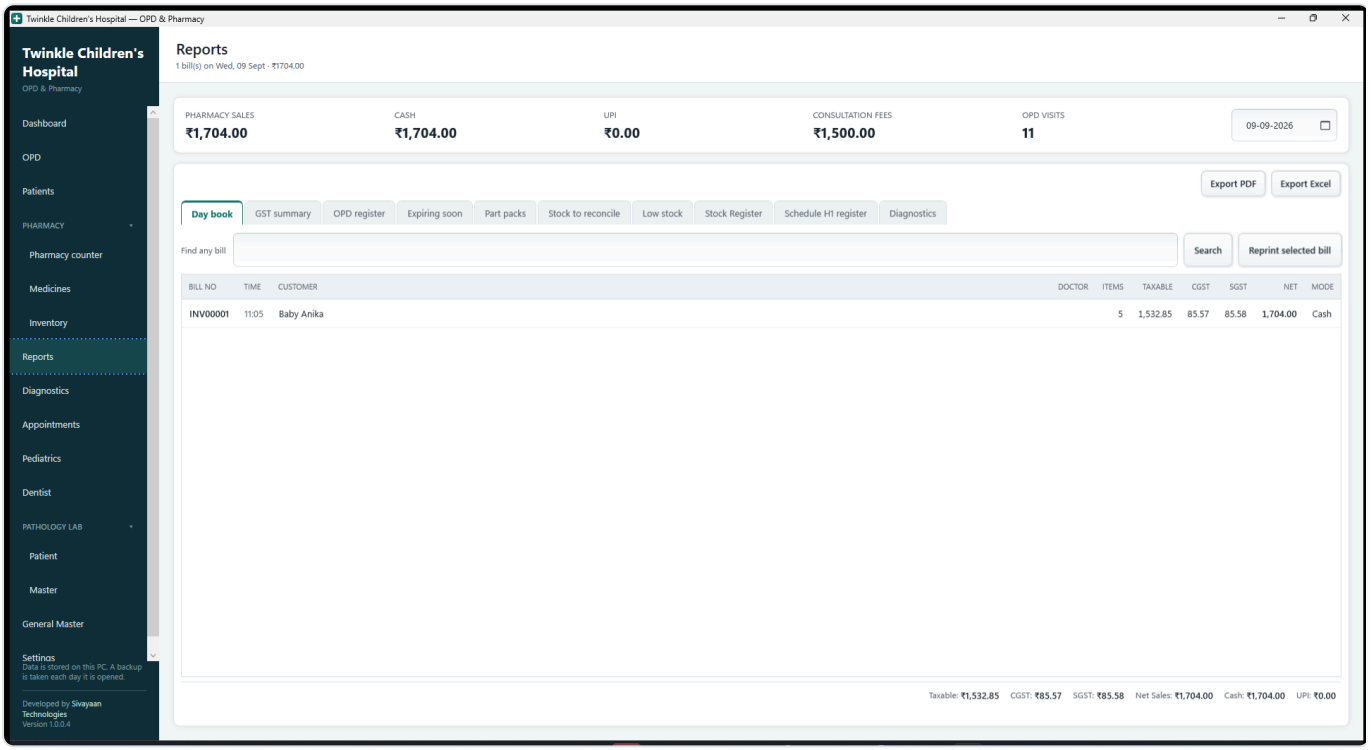
Vaccination history and **Growth chart** — every dose given and every measurement recorded for this child on Pediatrics, most recent included. Both shown only once that module is on.



> This is where you go when someone returns weeks later having lost a receipt.

---

13. Reports



Across the top for the chosen period: pharmacy sales, cash, UPI, consultation fees collected, and OPD visits.

One period for the whole screen

From and **To** at the top right set the period for every report that is about a stretch of time: **Day book**, **GST summary**, **OPD register**, **Schedule H1 register** and **Diagnostics**. They open on today to today. The heading, the exported PDF and Excel headers and the file names all carry the period, as `DayBook_2026-10-01_to_2026-10-31.xlsx`.

Expiring soon, **Low stock** and **Stock Register** have no period, on purpose: they are a picture of the shelf right now, and no choice of dates could change what they say. Exports of those three are dated today.

From after To is accepted and read the right way round.

Tab	What it is for
Day book	Every bill in the period. **Find any bill** searches every date by bill number or customer name. **Reprint selected bill** prints it again
GST summary	Taxable value, CGST and SGST grouped by rate — what a return needs
OPD register	Every visit in the period, diagnosis, fee, and whether it was paid. Age prints as 4y 3m; the Excel export keeps years and months as two numeric columns so they sort
Expiring soon	Batches within 90 days of expiry. Return these to the distributor
Part packs	Opened strips with less than a full pack left — sell these first, or write them off deliberately under Inventory → Correct the stock count
Stock to reconcile	Everything put on the shelf at the counter with no supplier bill behind it. See below
Low stock	Anything at or below its reorder level
Stock Register	Every batch on hand, searchable, with running totals for products, batches, units, cost value and MRP value
Schedule H1 register	Statutory record of H1 sales, by date range, with a running total. **Keep for three years**
Diagnostics	The period's diagnostic bills, with **Reprint selected bill**. Only shown once that module is on

Stock to reconcile

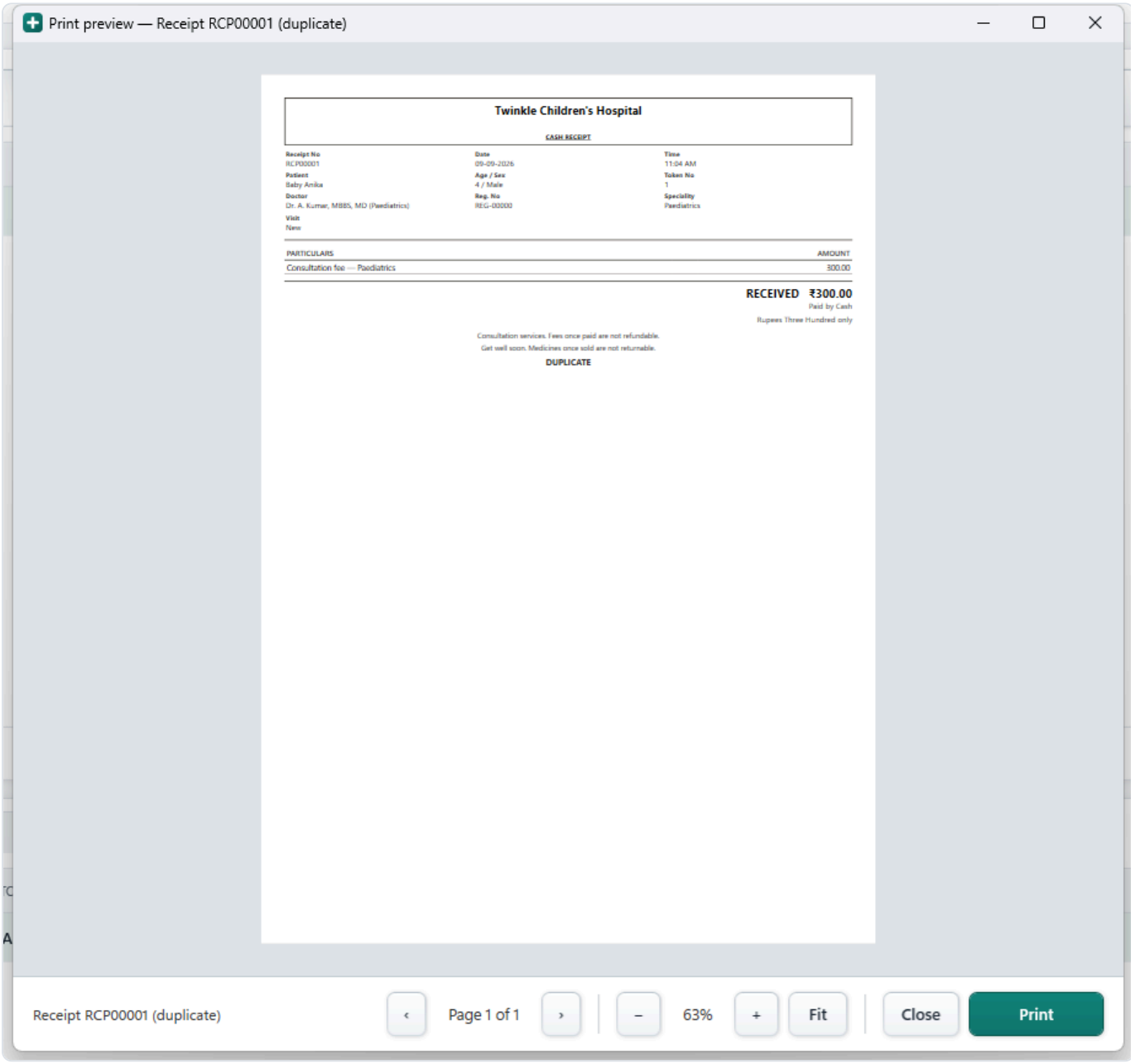
When the counter adds stock to serve a patient, there is no supplier bill behind it yet. Until there is, purchases will not tie out against sales — so every one of those entries is listed here with the date, medicine, batch, quantity on hand, MRP and the rate paid.

Work through it whenever it suits you. For each row, find the supplier bill it belongs to and check the quantity and the rate against it. A batch number starting `CTR-` was allocated by the system rather than read off the pack, so those are the ones worth checking against the real batch number.

The list is not a warning and does not have to be empty. It is a worklist.

14. Printing

Everything previews before any paper moves.



Print sends it. **Close** goes back. If no printer is set up the application says so plainly instead of failing, and you can still preview.

The preview opens with the whole sheet fitted to the window, so you can see the page the way it will come out of the printer before deciding.

Control	What it does
+ / **–**	Zooms in and out. Also **Ctrl** with the mouse wheel, or **Ctrl +** and **Ctrl –**

Control	What it does
Fit	Back to the whole page in view. Also **Ctrl 0**
‹ / ›	Moves between pages on a document that runs to more than one. Greyed out when there is only one page

Zoomed in past the window, the sheet scrolls — useful for checking a batch number or a total without printing a test copy. Resizing the window re-fits the page until you zoom yourself, after which it leaves your zoom alone.

Document	Number	Print it from
Tax invoice (medicines)	`INV00001`	Counter · Reports day book · patient record
Consultation receipt	`RCP00001`	OPD tile · patient record
Prescription	`V00001`	Consultation · OPD tile · patient record

Anything can be reprinted at any time. A reprint is stamped **DUPLICATE** so it cannot be mistaken for the original.

What the consultation receipt carries

Two rows of four, and nothing in them left blank:

Receipt No	**Date & time**	**Visit** — New or Review	**Token No**
Patient	**Age / Sex**	**Doctor**, with degrees	**Reg. No**

Then the fee line, the amount, the payment mode and the amount in words. A first visit with an OPD validity window also prints the date the cover runs to — see section 2.

The **speciality** is not a label of its own: it already names the consultation on the fee line (*Consultation fee — Paediatrics*), and a second copy of it in the identity block earned nothing.

The **visit number and patient number are not printed**. Both are internal references nobody at the counter can act on; the receipt number is the one to quote.

A **review** — a return inside the doctor's OPD validity window — prints the same receipt, with three differences:

- **Visit** reads **Review** instead of New.
- The amount is **₹0.00**, said out loud rather than left blank. A receipt with no figure on it invites exactly the question the figure answers.
- It is given **no receipt number of its own**. **Receipt No** shows the number the fee was actually taken on. Burning an `RCP` number on a nil amount would leave a hole in the day when the collection is reconciled — so a review and the visit it rides on show the same number, and it is the **Visit** line and the nil amount that tell the two apart.

15. Worked examples

Real situations, start to finish, with the numbers.

Example 1 — Nine tablets out of a strip of ten

A mother wants only nine Paracetamol. A strip holds ten and costs ₹30.

Step	What you do
1	**Pharmacy counter** → type `parac`
2	Click **Paracetamol 500mg**. It says "*590 tablets in stock · ₹3.00 each*"
3	**How many** `9` , **of** `tablets`
4	**Add to bill**

What you get

- Bill line: `0 × 10 TAB + 9 tablets` , amount **₹27.00**
- Shelf: 590 → 581 tablets
- The opened strip has one tablet left in it, and the next customer gets that first
- The search box empties, ready for the next medicine. The bill keeps the line

> **Why not ₹270?** Because the medicine says ten tablets per pack, so a tablet costs ₹3.00, not ₹30.00. If it charged ₹270, that medicine has **Units in one pack** set to 1 — see task P.

Example 2 — Two whole strips

Same medicine, but the customer wants two full strips.

How many `2` , **of** `strips of 10` → 20 tablets, **₹60.00**.

A full strip always costs exactly the MRP printed on it. Two strips is exactly twice that — never ₹59.90 or ₹60.10.

Example 3 — The medicine is found but has no stock

A prescription asks for Cetirizine syrup. The counter shows it, greyed, with "out of stock" — but you can see the bottles on the shelf.

This is the commonest situation in a new system: the delivery arrived and nobody entered it.

Step	What you do
1	Click the medicine anyway. The summary reads "*Cetirizine 10mg · out of stock*"
2	Click **Stock came in — add it**
3	**Packs on the shelf** `5`, **MRP per pack** `85.00`
4	Leave batch and expiry blank if you do not have them
5	**Add to shelf**

What you get

- Five bottles on the shelf, sellable immediately
- A batch numbered `CTR-260727-1432` — allocated, not printed on the pack
- A line in **Reports** → **Stock to reconcile** until the supplier bill arrives

You are back on the same bill. Nothing was lost.

Example 4 — The doctor prescribes something you do not stock

Dolo syrup, which the shop has never carried.

The doctor types the name and **does not** pick anything from the list. The hint reads "*Not in our pharmacy — it will be written on the prescription only*".

It is printed on the prescription and **never added to your medicine records**. At the counter, **Load prescription** names it: "*Not added: Dolo Syrup*". Tell the parent to buy it outside.

Example 5 — A course that spans two batches

Twenty tablets. The oldest batch holds seventeen.

You type `20`. The counter splits it and tells you:

> *Paracetamol: 20 from 2 batches — 15 from PC1234, 5 from PC1180.*

Two bill lines, because the batch number of what you actually hand over must be printed. The split falls at **fifteen, not seventeen**, so one whole strip is charged at the printed price and only five are loose — ₹116.65, the same as if one batch had covered it.

On the printed bill both lines sit under one **Paracetamol** heading, so it does not read as two separate items.

Example 6 — Nine tablets, but only six left

You type `9`. Nothing is added, and it says:

> *Only 6 tablets of Paracetamol 500mg left to sell.*

Sell the six, or add stock. It never quietly sells you fewer than you asked for.

Example 7 — A sealed pack that cannot be split

ORS sachets, sold as a box of ten, "Sell loose units" unticked.

You type `7` and it refuses:

> *ORS Powder is not sold loose — it goes out in whole packs of 10. Enter 10 for 1 pack.*

Untick that box on any medicine that must leave the shop whole.

Example 8 — A whole visit

Aarav, four years old, fever.

Step	Screen	What happens
1	OPD → **+ New visit**	Booked, token 4
2	Tile → **Fee**	₹300 taken, receipt printed
3	Tile → **Consult**	Fever, viral fever, three medicines prescribed
4	Counter → **Load prescription**	Paracetamol added; Cetirizine reported *no stock*; Dolo reported *not added*
5	**Stock came in — add it**	Five bottles of Cetirizine on the shelf
6	**Load prescription** again	Both now on the bill
7	**Save & print bill**	9 tablets + 1 bottle = **₹112.00**

The parent buys the Dolo syrup outside. Everything else is on one bill, with batch numbers and expiry printed on it.

---

16. What the system will and will not do

Worth reading once. It is the difference between trusting a number and checking it.

Situations it handles for you

Situation	What happens
A child needs 9 tablets from a strip of 10	Priced per tablet, nine come off the shelf, and the bill shows `0 × 10 TAB + 9`
A course needs 20 but the oldest batch holds 15	Split across two batches, two lines, both batch numbers on the bill. Nearest expiry goes first
A course needs 20 and there are only 12	It bills 12 and says "*Short: Amoxicillin (12 of 20)*". It never quietly bills less without telling you
A strip of 15 and a strip of 10 of the same drug	Each batch prices against the pack it actually came in. Old stock is never repriced
A medicine is prescribed that you do not stock	Written on the prescription, named at the counter, never added to your records
A medicine has run out mid-queue	Add it from the counter — see section 10
A doctor requests a lab test during consultation	**Load diagnostic tests** at the Diagnostics desk pulls it in with the patient already chosen — see section 11
A patient's age is not known exactly	Enter a date of birth instead and age is worked out and kept current, not typed once and left to go stale
Stock is expired	Never dispensed. It stays on the shelf listing so you can see it and return it
A sealed pack that cannot be split	Untick **Sell loose units** and the counter insists on whole packs, telling you the number to type
A Schedule H1 medicine	Cannot be saved without the prescribing doctor. It goes in the register automatically
Two people on one phone number	Both appear when you search. You pick which child is here
Two patients registered one after the other	The screen clears after each save, so the second is a new record. It cannot overwrite the first
A batch expiring within a month	Still sold first, and the counter says so before the customer leaves — batch, month and days remaining
The same supplier bill loaded twice	Refused. The invoice number is what stops it
The same batch delivered again	**Added** to what is there. Never replaced
The count on the shelf is wrong	Correct it with a reason. Every correction is recorded with was, now and why
A printer is not attached	Everything previews on screen first. Nothing is lost by not printing
The power goes off mid-bill	Nothing is half-saved. A bill either exists completely or not at all

Situation	What happens
Something unexpected fails	It says so in plain words and keeps running. Your saved data is safe

Situations it does not handle — check these yourself

Situation	What you need to know
A customer returns medicine	**There are no returns in this version.** Do it on paper and correct the stock count with reason *Other* and a note
Part payment, or paying later	Not supported, on purpose. A bill is paid in full when it is saved
Purchases balanced against sales	Will **not** tie out while anything sits in **Reports → Stock to reconcile**. That list is the gap, and it is deliberate
The rate paid on counter-added stock	Defaults to zero, so anything added that way looks like pure margin until you fill the real rate in
No discounts	The counter charges MRP. There is no discount box, by choice
A batch more than a month from expiry	Sold first, correctly, but not flagged. Watch **Expiring soon** for what to return
Two people billing at once	One PC, one till. This is not built for two counters on the same data
Changing units per pack after stock exists	Offered as a re-count, and you should accept it. Declining leaves the shelf being sold by the pack
A medicine deleted or made inactive with stock on it	It disappears from the counter but the stock is still counted in reports
Anyone can do anything	There are no logins or permissions. Everyone using the PC has the same rights

> ### The one number to sanity-check daily
>
> **Reports → the totals along the top.** The day's takings in the drawer should match pharmacy sales plus fees collected. A bill line can no longer be repriced, so a difference means cash was taken out or a bill was not saved — both worth finding on the day rather than at month end.

17. When something goes wrong

The application does not close on an error

If something unexpected happens it tells you in plain words, writes the details to the log, and **keeps running**. A half-typed bill is not lost. You will see a message like:

> *Taking the fee could not be completed. The change could not be saved — it may already exist, or a required field is missing. Nothing was changed.*

Things it refuses on purpose

It says	Why
"Only 3 × 10 TAB + 4 left of …"	You cannot sell stock you do not have
"Batch … expired on …"	Expired medicine cannot be dispensed
"3 people match that. Select which one"	Siblings share a phone; the wrong child is worse than a second click
"This patient has visits on record"	Deleting them would orphan their bills
"Bill … was already received on …"	Importing twice would double your stock
"No MRP" during import	The counter prices from MRP and cannot sell without one
"Batch number is required"	It has to appear on the bill by law

Your data

Everything is in one file: `C:\ProgramData\TwinkleHMS\twinkle.db`. Copy that file and you have copied the clinic.

Backups are automatic — one per day into `C:\ProgramData\TwinkleHMS\backups`, keeping the last 14. That protects against mistakes, **not against the PC dying**. Copy the folder to a pen drive weekly.

The log is at `C:\HMS\Logs`, one file per day, 30 days kept. Settings has an **Open log folder** button and shows the exact path. Send that day's file when reporting a problem.

Changing where things are kept

`appsettings.json` sits next to the application and can be edited in Notepad. Restart the application afterwards.

```
{
  "LogDirectory": "C:\\HMS\\Logs",
  "DatabasePath": null,
  "BackupsToKeep": 14,
  "LogDaysToKeep": 30
}
```

Setting	Notes
`LogDirectory`	Where daily logs are written
`DatabasePath`	Full path to the database file. `null` uses `C:\ProgramData\TwinkleHMS\twinkle.db`
`BackupsToKeep`	Daily database backups kept before the oldest is deleted
`LogDaysToKeep`	Days of logs kept

Use double backslashes in paths, as above.

> If the folder cannot be written to — a locked-down PC, or a network drive that is offline — the application does **not** fail. It falls back to `C:\ProgramData\TwinkleHMS\logs`, and the first line of the log says which folder it wanted and which it used. The same applies to a settings file with a typo in it: built-in defaults are used and the log says so.

Not in this version

Sales returns and credit notes · purchase returns · inter-state IGST · e-invoicing · multi-terminal use · the Schedule X narcotic register.

Credit and part payment are not missing — they are deliberately absent. The clinic settles every bill in full at the counter, so there is no outstanding balance anywhere in the system and nothing to reconcile.

The GST arithmetic and the invoice layout are correct for a retail counter, but this is not a certified e-invoicing integration. Have a CA review the GST summary before it feeds a return, and confirm register formats with your local drug inspector.

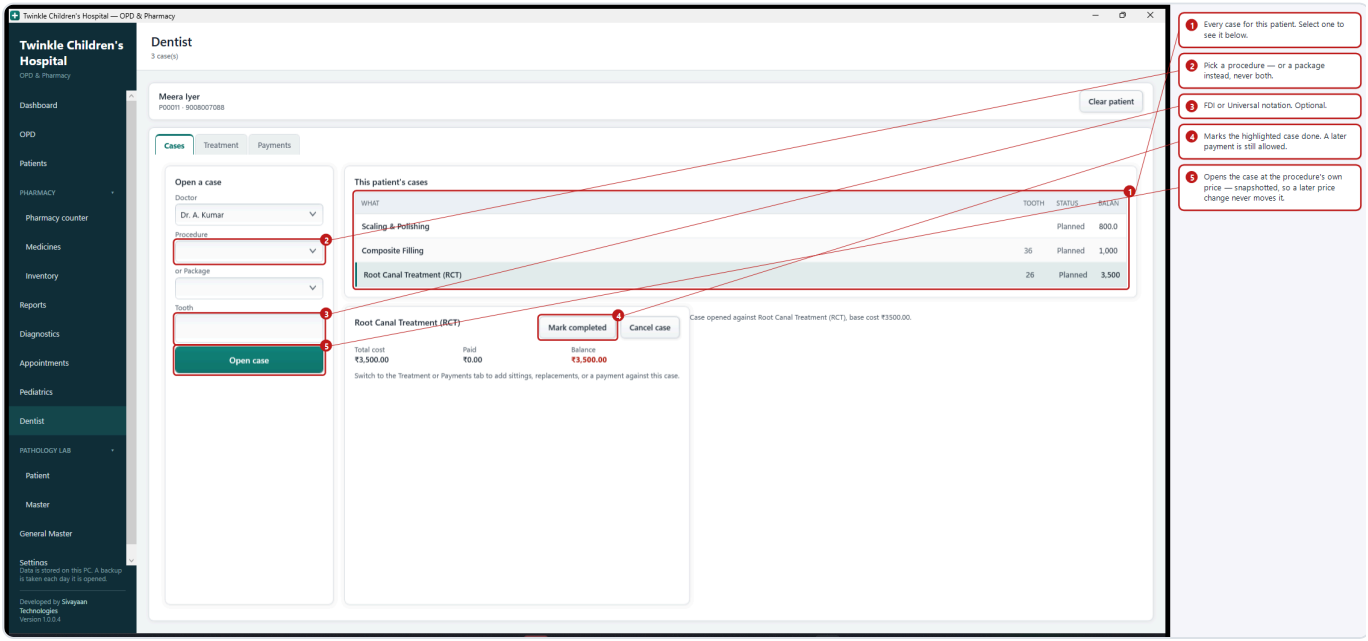
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18. The Dentist screen

Optional — off by default, same as Diagnostics. Switched on from **Settings** → **Features**, and once on it stays on; the nav button appears immediately, no restart needed. The patient panel sits above three tabs — **Cases**, **Treatment**, **Payments** — one task per tab, so nothing about sittings, replacements or payments is on screen while you are simply opening a case.

Cases

Pick a patient above, then open a case against a procedure or a package.

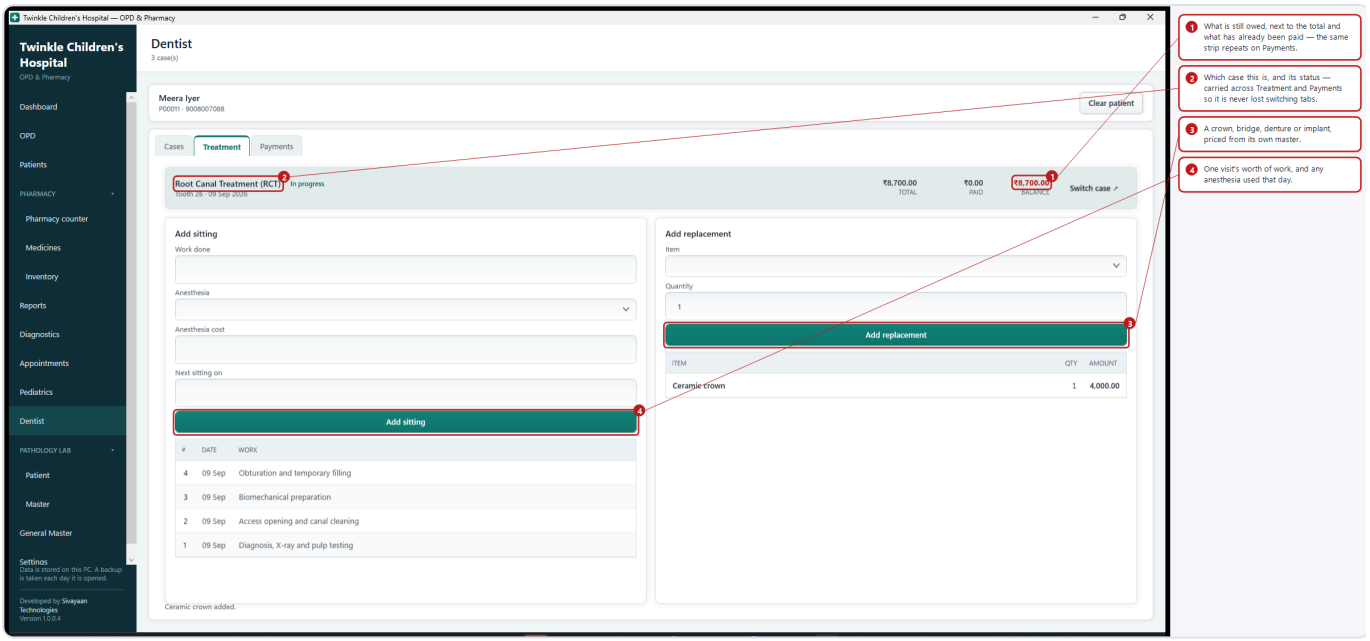


	Control	What it is for
1	**This patient's cases**	Every case on record for them. Select one to see it below
2	**Procedure**	What the case is opened against — or **or Package** below it instead, never both
3	**Mark completed**	Marks the selected case done. A later payment is still accepted — see **Cancel case** beside it for one that should not have been opened at all
4	**Tooth**	FDI or Universal notation. Optional
5	**Open case**	Opens the case at the procedure's own price — snapshotted, so a later master price change never moves it

Selecting a case shows its total, paid and balance, and a note pointing at the two tabs where the actual work happens — nothing about a sitting, replacement or payment can be entered from here.

Treatment

Sittings — what was done, and any anesthesia that day — and replacements — crowns, bridges, dentures, implants — against whichever case is selected on Cases.

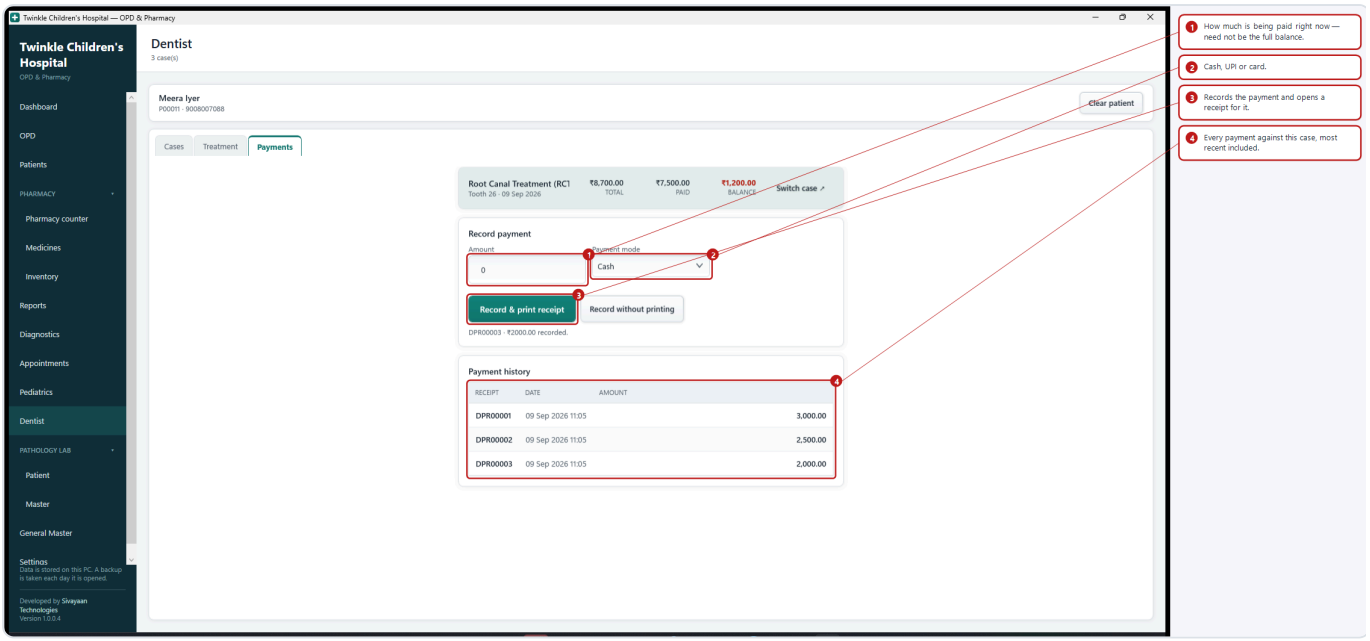


	Control	What it is for
1	**Balance**	What is still owed, next to the total and what has already been paid
2	**Case title**	Which case this is, and its status — the same strip repeats on Payments, so it is never lost switching tabs
3	**Add replacement**	Adds the picked item at its own unit cost × quantity
4	**Add sitting**	Logs the visit; the form clears itself once added, ready for the next one. Picking an anesthesia type fills in its default cost, which can still be changed

Switch case ↗, on the right of the strip, jumps straight back to Cases — for picking a different one without losing this one's progress, or simply for a look at the full case list again.

Payments

Record a payment against the same case, and see every payment already on it. Unlike the pharmacy counter, a dental case is not necessarily settled in one visit — a partial payment is normal, and the balance carries to the next one.



	Control	What it is for
1	**Amount**	How much is being paid right now — need not be the full balance
2	**Payment mode**	Cash, UPI or card
3	**Record & print receipt**	Records the payment and opens a receipt for it. **Record without printing** does the same without opening the preview
4	**Payment history**	Every payment against this case, most recent included

Why a strip, not the whole detail panel

Total, paid, balance, title and status used to sit in the middle of one crowded screen alongside sittings, replacements and payments, all visible at once whether or not they were being used. Splitting those onto their own tabs meant the case itself needed a way to stay in view without repeating the Cases tab's full card — the strip is exactly that: enough to know which case and how much is owed, nothing more, wherever it is needed.

The four masters this screen bills against — **Procedures**, **Dental Replacements**, **Anesthesia Types** and **Dental Packages** — live on the shared **General Master** screen, its own nav entry, alongside Pediatrics' Vaccine Master and the Procedure Master both specialties draw from.

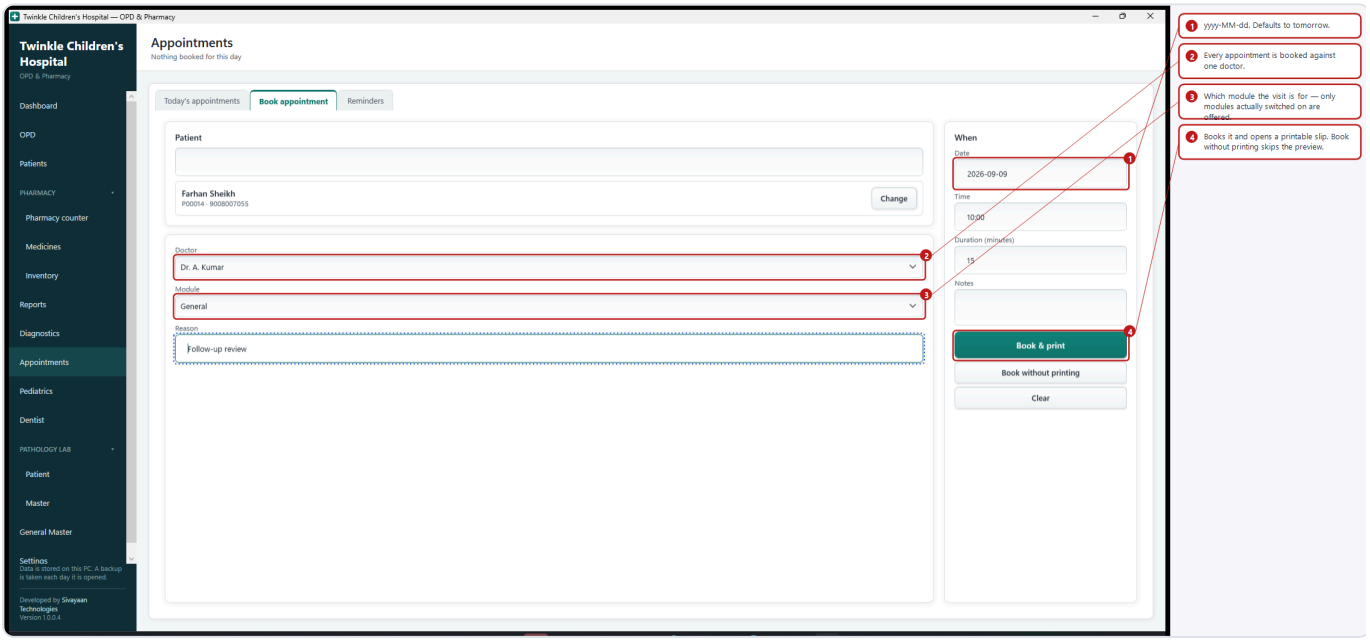
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19. Appointments

Optional — off by default, same as Diagnostics. Switched on from **Settings** → **Features**; the nav button appears immediately, no restart needed. Three tabs — **Today's appointments**, **Book appointment**, **Reminders**.

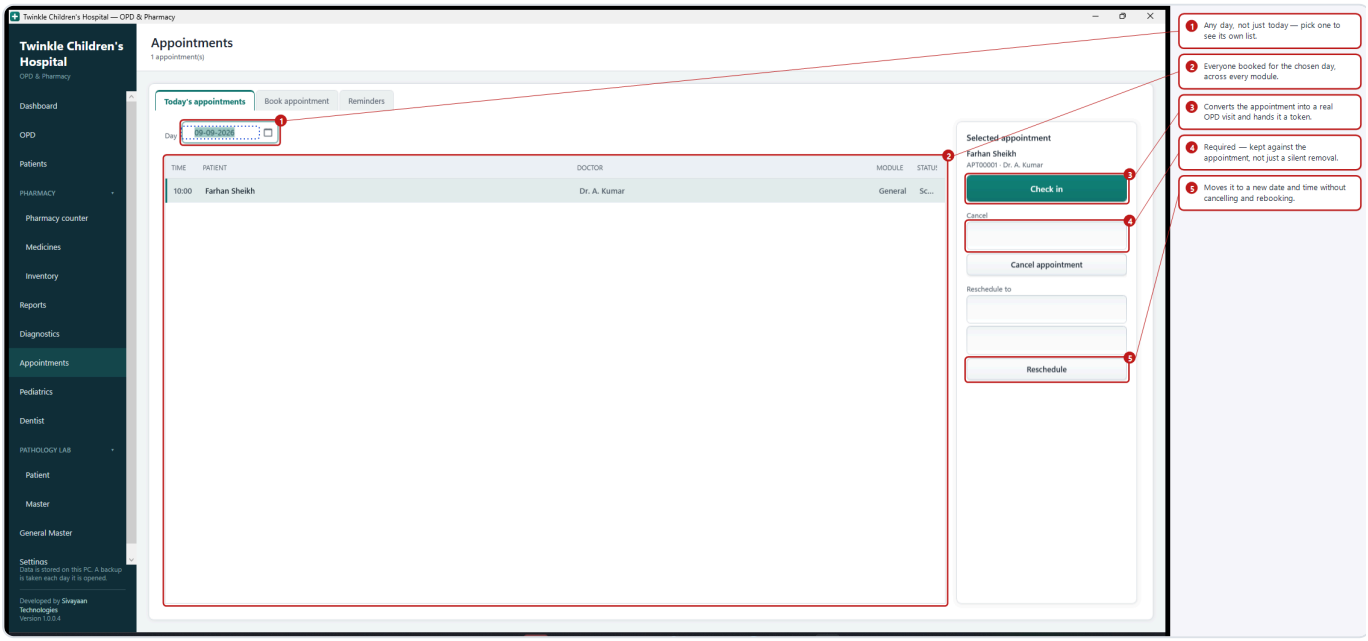
Book appointment

Pick a patient above the form the same way as everywhere else — search by name or phone, or **+ New patient** for someone not yet registered.



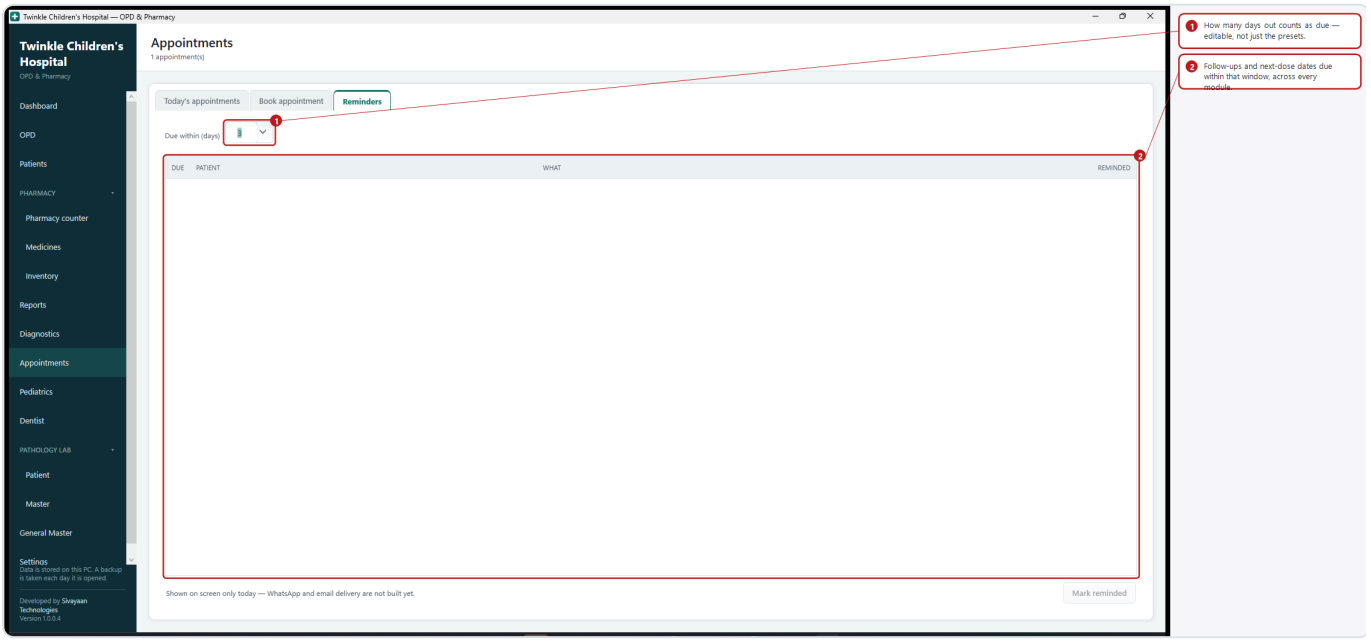
	Control	What it is for
1	**Date**	yyyy-MM-dd. Defaults to tomorrow
2	**Doctor**	Every appointment is booked against one doctor
3	**Module**	Which module the visit is for — General (OPD), Pediatrics, Dentist or Pathology Lab. Only modules actually switched on are offered
4	**Book & print**	Books it and opens a printable slip. **Book without printing** skips the preview

Today's appointments



	Control	What it is for
1	**Day**	Any day, not just today — pick one to see its own list
2	**The list**	Everyone booked for the chosen day, across every module
3	**Check in**	Converts the appointment into a real OPD visit and hands it a token — wired up for the General module only today. A Dentist or Pathology Lab appointment is still tracked here, but is checked in from its own screen
4	**Cancel**	A reason is required — kept against the appointment, not just a silent removal
5	**Reschedule**	Moves it to a new date and time without cancelling and rebooking

Reminders



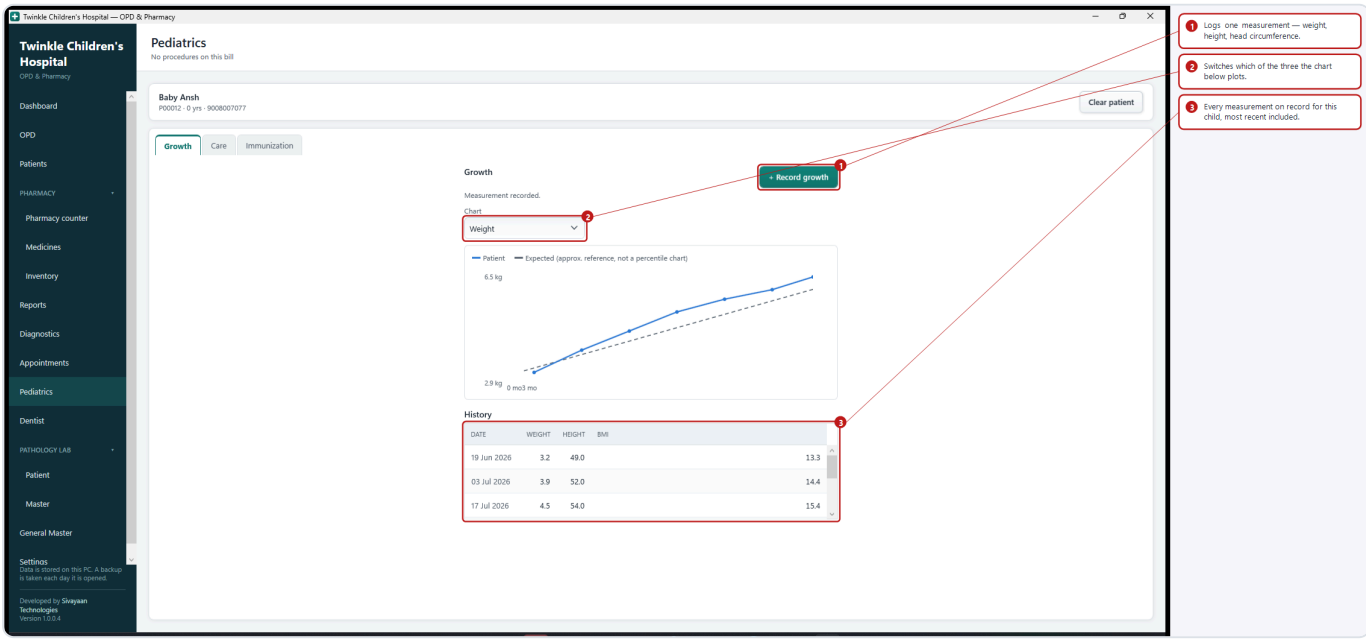
	Control	What it is for
1	**Due within (days)**	How many days out counts as due — editable, not limited to the presets
2	**The list**	Follow-ups and next-dose dates due within that window, across every module

Mark reminded records that the desk actually followed up — it does not send anything itself. WhatsApp and email delivery are not built yet; this list is for the desk to work through by phone.

20. Pediatrics

Optional — off by default, same as Diagnostics. Switched on from **Settings** → **Features**; the nav button appears immediately, no restart needed. The patient panel sits above three tabs — **Growth**, **Care**, **Immunization** — one task per tab.

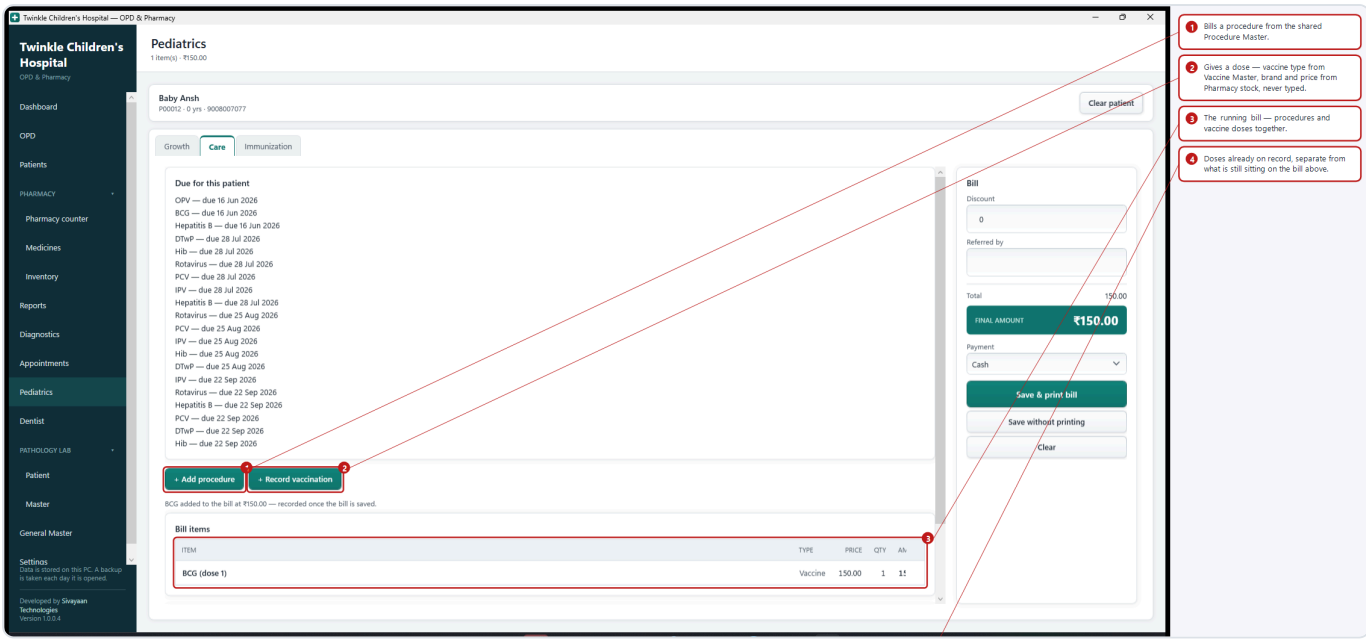
Growth



	Control	What it is for
1	**+ Record growth**	Logs one measurement — weight, height, head circumference
2	**Chart**	Switches which of the three the chart below plots, against an approximate reference line — not a percentile chart
3	**History**	Every measurement on record for this child, most recent included

Care

Procedure billing and vaccination, landing on the same running bill.



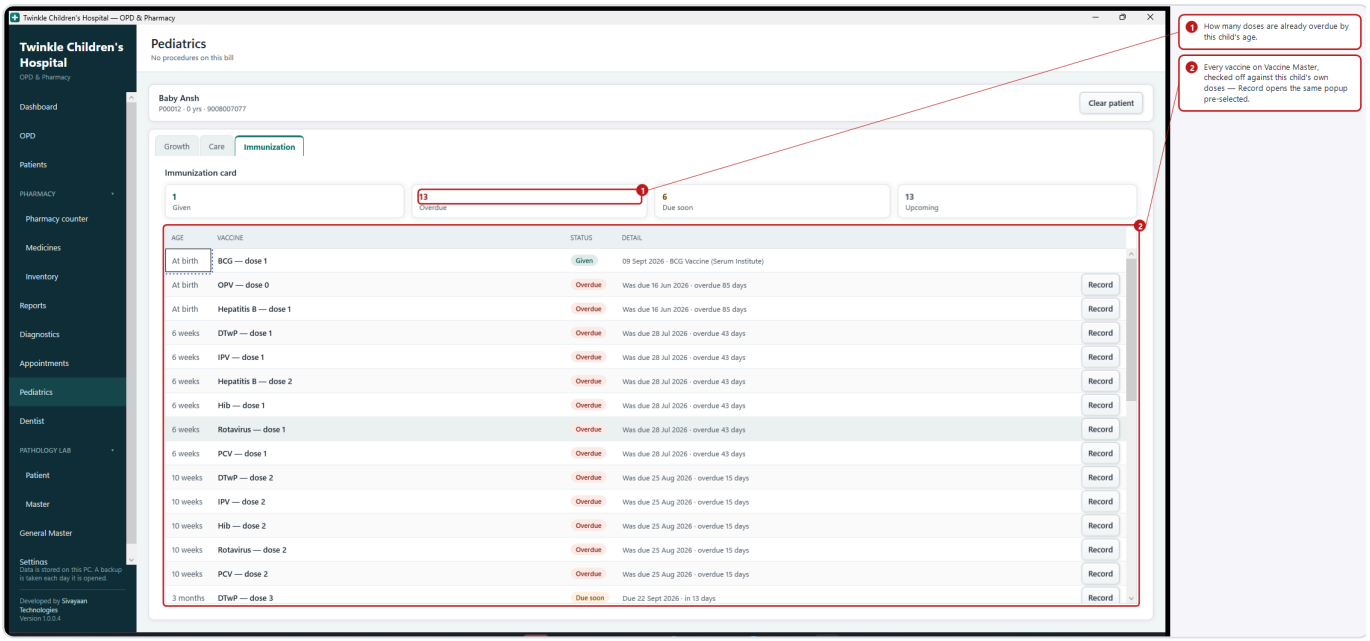
	Control	What it is for
1	**+ Add procedure**	Bills a procedure from the shared Procedure Master
2	**+ Record vaccination**	Gives a dose — vaccine type from Vaccine Master, brand and price from Pharmacy stock, never typed
3	**Bill items**	The running bill — procedures and vaccine doses together

Previous vaccination history, further down the same tab, lists doses already on record — separate from what is still sitting on the bill above — with its own **Print** button for a vaccination certificate and a **Clear patient** for moving straight to the next child.

> ### A dose is not "given" until the bill is saved
>
> Recording a dose here only adds it to the running bill, the same as adding a procedure. Nothing is written to this child's vaccination record until that bill is actually saved — clicking **Clear**, or simply moving on without saving, drops it with nothing left behind. A free dose still needs a saved bill, at ₹0, to actually count as given.

Immunization

This child's own doses, checked off against Vaccine Master's recommended ages.



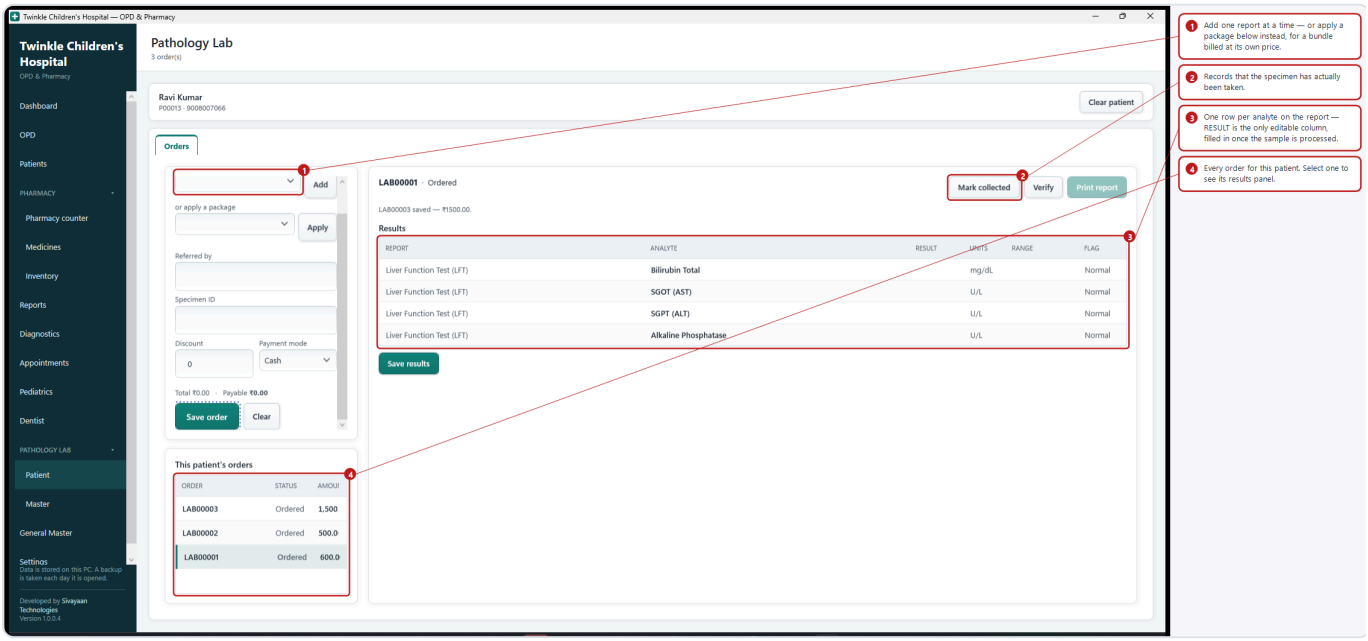
	Control	What it is for
1	**Given / Overdue / Due soon / Upcoming**	Counts across every vaccine on Vaccine Master for this child, worked out from their date of birth
2	**The card**	One row per vaccine and dose. **Record** — shown only on rows that still need it — opens the same popup as Care's Record vaccination, pre-selected to that row

> A patient with no date of birth on file has nothing to compare a recommended age against, so every not-yet-given row reads **Upcoming** regardless of the child's actual age — set a date of birth on the patient record to see real Overdue and Due soon rows.

21. Pathology Lab

Optional — off by default. Switched on from **Settings** → **Features**; the nav button appears immediately, no restart needed. The patient screen and its own master screen are separate nav entries, grouped under one collapsible **Pathology Lab** header in the sidebar — the same shape as Pharmacy.

Orders



	Control	What it is for
1	**Add report**	Add one report at a time — or **apply a package** below it instead, for a bundle billed at its own price
2	**Mark collected**	Records that the specimen has actually been taken
3	**Results**	One row per analyte on the report — RESULT is the only editable column, filled in once the sample is processed
4	**This patient's orders**	Every order for this patient. Select one to see its results panel

A saved order moves through **Ordered** → **Sample Collected** → **Result Entered** → **Verified** → **Completed**. **Save results** needs at least one row filled in; **Verify** needs at least one result saved first; **Print report** stays greyed out until the order is verified.

> A result outside its own reference range flags itself automatically — H or L beside the value — the moment it is saved, no separate step needed.

Master

Analyte, Report and Package masters, on their own nav entry so a large catalogue does not crowd the patient screen above.



	Control	What it is for
1	**Search**	By name or category
2	**+ New analyte**	Name, category, units, result type (numeric or text) and reference range
3	**The grid**	A set of common analytes is preloaded — hematology, biochemistry, thyroid, infection screens and more, editable from here

Report Master bundles analytes into one billable report — the CBC seen above, for instance, bundles Hemoglobin, Total Leukocyte Count and more. **Package Master** bundles reports into one flat-priced package, the same "price wins over the sum of its parts" rule Dentist packages already follow.

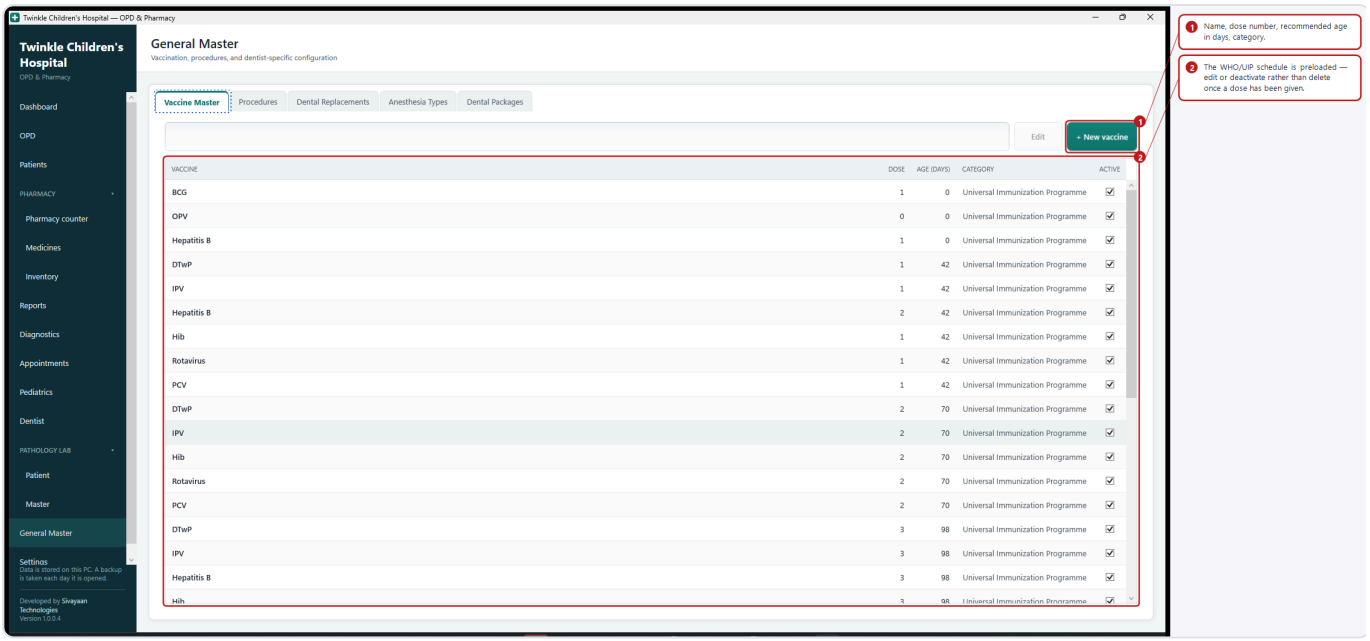
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22. General Master

Not a module of its own — the shared configuration behind Pediatrics and the Dentist screen, on one nav entry so a vaccine list and a procedure list are never maintained twice. Pathology Lab keeps its own separate Master screen (above) since its analyte/report/package shape shares nothing with Procedure.

Vaccine Master

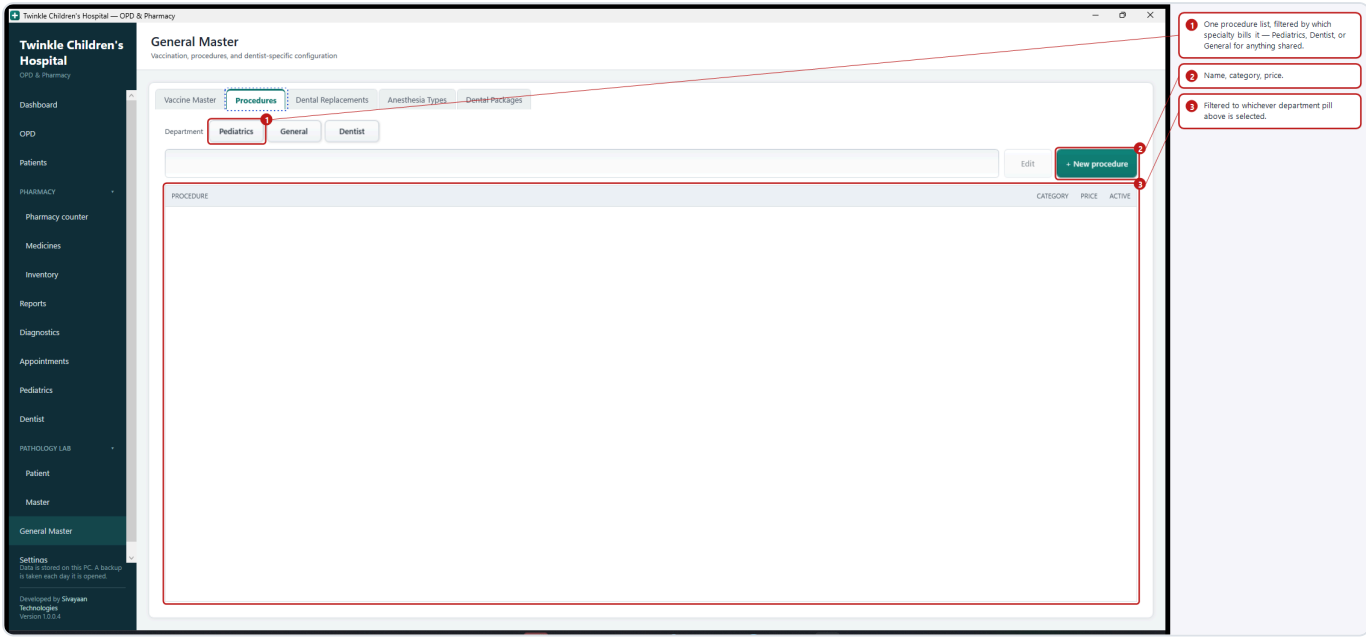
Only shown once Pediatrics is on.



	Control	What it is for
1	**+ New vaccine**	Name, dose number, recommended age in days, category
2	**The grid**	The WHO/UIP schedule is preloaded — edit or deactivate rather than delete once a dose has actually been given to somebody

Procedures

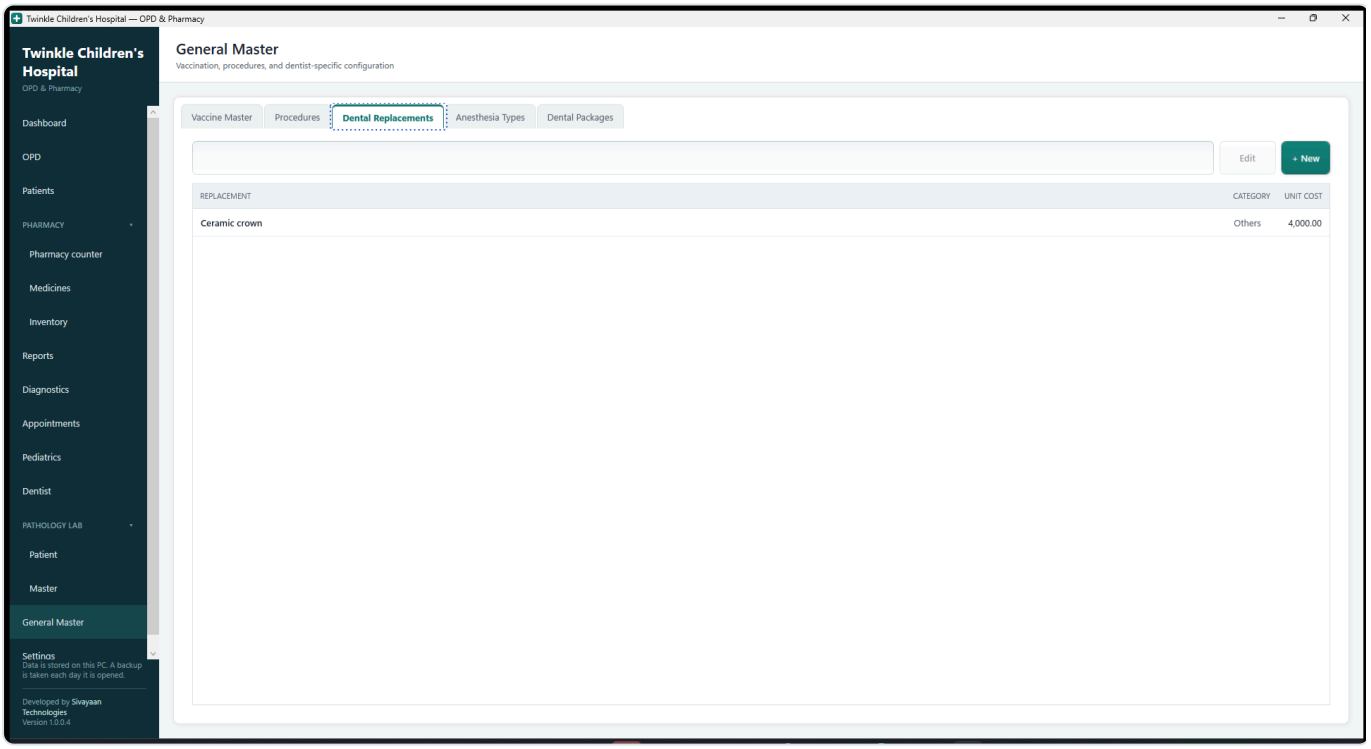
Shared by Pediatrics and Dentist rather than each keeping a separate list.



	Control	What it is for
1	**Department**	One procedure list, filtered by which specialty bills it — Pediatrics, Dentist, or General for anything shared between them
2	**+ New procedure**	Name, category, price
3	**The grid**	Filtered to whichever department pill above is selected

Dental Replacements, Anesthesia Types, Dental Packages

Only shown once Dentist is on — three more masters behind the same screen, each the same shape as Procedures above: a search, a **+ New** button, an editable grid.



Dental Replacements (crowns, bridges, dentures, implants) and **Anesthesia Types** are picked from directly on the Dentist screen's own Treatment tab. **Dental Packages** bundle procedures into one flat-priced package — see the Dentist screen for how a case is opened against either a procedure or a package.